

Antihistamine

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตับ/ไต	ข้อห้ามใช้/ควรระวัง
Chlorpheniramine	Upper respiratory tract conditions: Temporary relief of symptoms (runny nose; sneezing; itching of the eyes, nose, or throat) associated with the hay fever (allergic rhinitis) and other upper respiratory tract conditions.	Upper respiratory tract conditions: Immediate release: Oral: 4 mg every 4 to 6 hours; do not exceed 24 mg/24 hours. Extended release: Oral: 12 mg every 12 hours; do not exceed 24 mg/24 hours. Dosing: Older Adult Avoid use Administration: Adult Oral: May be administered with food or water. • Timed release oral forms are to be swallowed whole, not crushed or chewed. • Administer liquid formulations with an accurate measuring device; do not use a household teaspoon (overdosage may occur).	Dosage guidance: Safety: Safety and efficacy for the use of cough and cold products in infants and young children are limited; the AAP warns against the use of these products for respiratory illnesses in infants and young children; the FDA does not recommend OTC use in infants and children <2 years of age due to the risk of serious and life-threatening adverse effects (including death) and recommends to use with caution in pediatric patients ≥2 years of age Administration: Pediatric Oral: Immediate release: • Tablet: May be administered with food or water. • Syrup: Administer liquid formulations with an accurate measuring device; do not use a household teaspoon (overdosage may occur). Extended release: • Tablet: Swallow whole; do not crush or chew. May be administered with food or water.	Altered kidney function: • eGFR >30 mL/minute/1.73 m ² : No dosage adjustment necessary • eGFR ≤30 mL/minute/1.73 m ² : No initial dosage adjustment necessary; however, greatly prolonged half-lives in patients with severe kidney disease have been reported; use with caution Dosing: Liver Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling; however, chlorpheniramine is metabolized via the liver; therefore, a dose adjustment may be necessary.	Contraindications OTC labeling: When used for self-medication, do not use to make a child sleep. Avoid use in patients with high-risk occupations (eg, pilots, bus drivers) or who may be more prone to anticholinergic effects (eg, older adults) Warnings/Precautions • CNS depression: May cause CNS depression, which may impair physical or mental abilities; patients must be cautioned about performing tasks which require mental alertness (eg, operating machinery or driving). Special populations: • Pediatric: Antihistamines may cause excitation in young children. Other warnings/precautions: • Self-medication (OTC use): When used for self-medication (OTC), notify health care provider prior to use if you have breathing problems (eg, chronic bronchitis, emphysema), glaucoma, difficulty in urination due to enlargement of the prostate gland, or are currently taking sedatives, tranquilizers. Warnings: Additional Pediatric Considerations Safety and efficacy for the use of cough and cold products in pediatric patients <4 years of age is limited; the AAP warns against the use of these products for respiratory illnesses in young

					children. Serious adverse effects including death have been reported. Many of these products contain multiple active ingredients, increasing the risk of accidental overdose when used with other products. The FDA does not recommend OTC uses for these products in pediatric patients <2 years of age and recommends to use with caution in pediatric patients ≥2 years of age. Health care providers are reminded to ask caregivers about the use of OTC cough and cold products in order to avoid exposure to multiple medications containing the same ingredient (AAP 2018; CDC 2007; FDA 2017; FDA 2018).
Brompheniramine	Brompheniramine is an antihistamine that is used to treat sneezing, runny nose, and itchy or watery eyes caused by allergies, hay fever, and the common cold. Ref : drugs.com	Usual Adult Dose for: Cold Symptoms Maintenance dose: 20 mL orally every 4 hours Maximum dose: 6 doses orally in 24 hours Ref : drugs.com ขนาด บรอมเฟนิรามีน สำหรับผู้ใหญ่ อาการของโรคหวัด • ยาออกฤทธิ์ทันที 4 มก. ถึง 8 มก. รับประทานทุกๆ 6 ชั่วโมงเท่าที่จำเป็น ระยะเวลาในการรักษานั้นแตกต่างกันตามแต่ละบุคคล ผู้ป่วยจำนวนมากอาจต้องการยาเพียงแค่วันละ 2 ครั้ง • ยาออกฤทธิ์นาน 6 มก. ถึง 12 มก. รับประทานวันละ 2 ครั้งเท่าที่จำเป็น ระยะเวลาในการรักษานั้นแตกต่างกันตามแต่ละบุคคล ผู้ป่วยจำนวนมากอาจจะต้องการยาแค่วันละครั้ง ก่อนนอนเพื่อหลีกเลี่ยงอาการง่วงซึม ขนาดยาสำหรับรับประทานสูงสุด 24 มก. ต่อวัน ผู้สูงอายุ (60 ปีขึ้นไป) นั้นมักจะมีอาการแทรกซ้อนได้มากกว่าควรใช้ด้วยความระมัดระวังหากจำหน่าย	Usual Pediatric Dose for Cold Symptoms • 6 to less than 12 years: Maintenance dose: 10 mL orally every 4 hours Maximum dose: 6 doses orally in 24 hours • 12 years and older: Maintenance dose: 20 mL orally every 4 hours Maximum dose: 6 doses or	Renal Dose Adjustments Data not available Liver Dose Adjustments Data not available	Precautions Safety and efficacy have not been established in patients younger than 6 years. Warnings Ask a doctor or pharmacist before using brompheniramine if you have health problems or use other medications, or if you are pregnant or breastfeeding. Do not use cold or allergy medicine to make a child sleepy. Before taking this medicine You should not use brompheniramine if you are allergic to it, or if you have: • severe or uncontrolled high blood pressure (hypertension) • severe coronary artery disease (clogged arteries) • a stomach ulcer • narrow-angle glaucoma • if you are unable to urinate or • if you are having an asthma attack. Do not use

		ยานี้ให้แก่วัยสูงอายุ https://hellokhunmor.com/ยาและอาหารเสริม/บรอมเฟนิรามีน-brompheniramine/			brompheniramine if you have used an MAO inhibitor in the past 14 days. A dangerous drug interaction could occur. MAO inhibitors include isocarboxazid, linezolid, methylene blue injection, phenelzine, rasagiline, selegiline, tranylcypromine, and others. Ask a doctor or pharmacist if this medicine is safe to use if you have: • breathing problems • a bladder obstruction or other urination problems • an enlarged prostate • a stomach ulcer • a blockage in your digestive tract (stomach or intestines) • glaucoma • heart disease • high blood pressure • a thyroid disorder • liver or kidney disease • diabetes • seizures
Diphenhydramine	Use: Labeled Indications Symptomatic relief of allergic symptoms caused by histamine release, including nasal allergies and allergic dermatosis	Common cold symptoms (eg, rhinitis, rhinorrhea, sneezing) or upper airway cough syndrome: Note: Used in combination with a decongestant; adverse effects of monotherapy likely outweigh benefits Oral: 25 mg every 4 to 6 hours or 50 mg every 6 to 8 hours as needed (in combination with a decongestant); doses at the higher end of the dosing range may not be tolerated. Dosing: Older Adult : Avoid use; may be appropriate for acute treatment of severe allergic reaction	Allergic rhinitis/rhinoconjunctivitis (including upper respiratory allergies or hay fever): Children 2 to <6 years: Limited data available: Oral: 6.25 mg every 4 to 8 hours as needed Children ≥6 to <12 years: Oral: 12.5 to 25 mg every 4 to 8 hours as needed Children ≥12 years and Adolescents: Oral: 25 to 50 mg every 4 to 8 hours as needed Weight-directed dosing: Infants ≥9 kg, Children, and Adolescents: Oral: 5 mg/kg/day in equally divided doses every 6 to 8 hours; age-related maximum single doses: <6 years: 6.25 mg/dose; 6 to 11 years: 25 mg/dose; ≥12 years: 50 mg/dose (Ref).	Altered kidney function: IM, IV, Oral: No dosage adjustment necessary for any degree of kidney function (little excreted unchanged in urine) Liver impairment prior to treatment initiation: Child-Turcotte-Pugh class A to C: IM, IV, Oral: No dosage adjustment necessary, use lowest effective dose and as needed dosing only	Contraindications OTC labeling: When used for self-medication, do not use in children <6 years, to make a child sleep, or with any other diphenhydramine-containing products (including topical products) Warnings/Precautions • CNS depression: May cause CNS depression, which may impair physical or mental abilities; patients must be cautioned about performing tasks which require mental alertness (eg, operating machinery or driving). Disease-related concerns: • Asthma: Use with caution in patients with a history of asthma. • Cardiovascular disease: Use with caution in patients with cardiovascular

					disease (including hypertension and ischemic heart disease). • Increased intraocular pressure/glaucoma: Use with caution in patients with increased intraocular pressure or angle-closure glaucoma. • Prostatic hyperplasia/urinary obstruction: Use with caution in patients with prostatic hyperplasia, bladder neck obstruction, and/or GU obstruction. • Pyloroduodenal obstruction: Use with caution in patients with pyloroduodenal obstruction (including stenotic peptic ulcer). • Thyroid dysfunction: Use with caution in patients with thyroid dysfunction. Special populations: • Pediatric: Antihistamines may cause excitation in young children. Toxicity (overdose) in pediatric patients may result in hallucinations, convulsions, or death; neonates and young children are highly sensitive to depressive effects of diphenhydramine; use is contraindicated in neonates and premature infants.
Cyproheptadine	Use: Labeled Indications Allergic conditions: Perennial and seasonal allergic rhinitis; vasomotor rhinitis;	Allergic conditions: Oral : Initial: 4 mg three times daily; maintenance: 4 to 20 mg/day in divided doses; maximum: 0.5 mg/kg/day; some patients may require up to 32 mg/day for adequate control of symptoms. Dosing: Older Adult Avoid use	Allergic conditions (nonacute): Weight-directed or BSA-directed dosing: Children ≥2 years and Adolescents: Oral: 0.25 mg/kg/day or 8 mg/m²/day in 2 to 3 divided doses. Fixed dosing: • Children 2 to <7 years: Oral: 2 mg every 8 to 12 hours; maximum daily dose: 12 mg/day. • Children ≥7 years and Adolescents <15 years: Oral: 4 mg every 8 to 12 hours; maximum daily dose: 16 mg/day. • Adolescents ≥15 years: Initial: Oral: 4 mg every 8 hours; titrate to effect; usual range: 12 to 16	Dosing: Kidney Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling. However, elimination is diminished in renal insufficiency. Dosing: Liver Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling.	Avoid use of cyproheptadine in patients with high-risk occupations (eg, pilots, bus drivers) or who may be more prone to anticholinergic effects (eg, older adults)

			mg/day although some patients may require up to 32 mg; maximum daily dose: 0.5 mg/kg/day.		
Hydroxyzine	Pruritus: Management of pruritus due to allergic conditions (eg, atopic and contact dermatoses) and histamine-mediated pruritus.	Pruritus: • Manufacturer's dosing: Oral: 25 mg 3 to 4 times daily as needed. • Nightly dosing: Oral: Due to its sedating properties, some experts recommend 25 to 50 mg once daily as needed at bedtime. Dosing: Older Adult Avoid use Administration: Adult Oral: Administer without regard to food.	Pruritus; associated with allergic conditions or chronic urticaria: Age-directed dosing: • Children <6 years: Oral: 12.5 mg 3 to 4 times daily • Children ≥6 years and Adolescents: Oral: 12.5 to 25 mg 3 to 4 times daily Weight-directed dosing: Children and Adolescents: • Patient weight ≤40 kg: Oral: 2 mg/kg/day divided every 6 to 8 hours as needed; maximum dose: 25 mg/dose. • Patient weight >40 kg: Oral: 25 to 50 mg once daily at bedtime or twice daily Administration: Pediatric Oral: May be administered without regard to food.	Altered kidney function: Oral, IM: • CrCl ≥50 mL/minute: No dosage adjustment necessary • CrCl 10 to <50 mL/minute: Administer ~50% of usual dose • CrCl <10 mL/minute: Administer ~25% to 50% of usual dose Liver impairment prior to treatment initiation : Initial or dose titration in patients with preexisting liver cirrhosis: Child-Turcotte-Pugh class A to C: IM, Oral: Use minimum effective dose, not to exceed a total maximum daily dose of 25 mg/day	Contraindications Hypersensitivity to hydroxyzine or any component of the formulation; early pregnancy; prolonged QT interval. Additional contraindications: Oral: Hypersensitivity to cetirizine or levocetirizine. Warnings/Precautions Concerns related to adverse effects: • QT prolongation/torsades de pointes: Oral hydroxyzine is contraindicated in patients with a prolonged QT interval. Disease-related concerns: • Glaucoma: Use with caution in patients with narrow-angle glaucoma; condition may be exacerbated by cholinergic blockade. Screening is recommended. • Prostatic hyperplasia/urinary stricture: Use with caution in patients with prostatic hyperplasia and/or urinary stricture. • Respiratory disease: Use with caution in patients with asthma or chronic obstructive pulmonary disease (COPD). Special populations: Older adult: May cause over sedation in older adults; avoid use.
Cetirizine	Use: Labeled Indications Oral: Allergic rhinitis: Relief of symptoms associated with allergic rhinitis.	Allergic rhinitis or conjunctivitis: Oral: 10 mg once daily; alternatively, may be given as needed 2 to 5 hours before exposure to allergen, although this may be less effective than daily administration OTC labeling (patient-guided therapy for symptoms of hay fever or other upper	Allergic rhinitis or conjunctivitis or rhinoconjunctivitis (including upper respiratory allergies or hay fever): • Infants ≥6 months: Oral: 2.5 mg once daily. • Children <2 years: Oral: Initial: 2.5 mg once daily; dosage may be increased to 2.5 mg twice daily. • Children 2 to <6	Altered kidney function: Oral: Note: Dose recommendations are based on usual doses of 5 to 10 mg once daily. • CrCl >31 mL/minute: No dosage adjustment necessary. • CrCl 11 to ≤31 mL/minute: 5 mg once daily. • CrCl ≤10	Contraindications Hypersensitivity to cetirizine, hydroxyzine, levocetirizine, or any component of the formulation. Warnings/Precautions Concerns related to adverse effects: • Pruritus: Rebound pruritus has been reported within

		respiratory allergies): 5 to 10 mg once daily (maximum: 10 mg/day). Dosing: Older Adult Upper respiratory allergies, urticaria: Oral: 5 mg once daily (maximum dose: 5 mg daily). The previously available prescription product recommended a maximum dose of 10 mg once daily in patients <77 years of age or 5 mg once daily in patients ≥77 years of age Administration: Adult • Oral: May be administered with or without food. • Chewable tablet: Chew tablet before swallowing; may be taken with or without water.	years: Oral: 2.5 to 5 mg once daily (Ref); may also administer as 2.5 mg twice daily • Children ≥6 years and Adolescents: Oral: 5 to 10 mg once daily. Administration: Pediatric • Oral: Administer without regard to food. • Chewable tablet: Chew or crush tablet completely before swallowing; may be taken with or without water. • Orally disintegrating tablet: Allow tablet to melt in mouth; may be taken with or without water. • Oral solution : Administer with an accurate measuring device; do not use a household teaspoon (overdosage may occur).	mL/minute: 5 mg once every 48 hours; may increase to 5 mg once daily based on tolerability and response for short-term use only (drug may accumulate with prolonged use at this dose) Liver impairment prior to treatment initiation: Child-Turcotte-Pugh class A to C: Oral: 5 mg once daily	several days after stopping cetirizine, usually after long-term (eg, months to years) use. Disease-related concerns: • Hepatic impairment: Use with caution; consider dosage adjustment. • Renal impairment: Use with caution; consider dosage adjustment. Concurrent drug therapy issues: • Sedatives: Effects may be potentiated when used with other sedative drugs or ethanol. Special populations: • Older adult: Use with caution in older adults; may be more sensitive to adverse effects. Warnings: Additional Pediatric Considerations Some dosage forms may contain propylene glycol; in neonates, large amounts of propylene glycol delivered orally, intravenously (eg, >3,000 mg/day), or topically have been associated with potentially fatal toxicities which can include metabolic acidosis, seizures, renal failure, and CNS depression; toxicities have also been reported in children and adults including hyperosmolality, lactic acidosis, seizures, and respiratory depression; use caution (AAP 1997; Shehab 2009).
Loratadine	Use: Labeled Indications Allergic rhinitis or conjunctivitis: Relief of nasal and non-nasal symptoms of seasonal allergies. OTC labeling: Patient-guided therapy for symptoms of hay fever or other upper respiratory allergies.	Allergic rhinitis or conjunctivitis: Oral: 10 mg once daily or 5 mg twice daily. OTC labeling (patient-guided therapy for symptoms of hay fever or other upper respiratory allergies): Oral: 10 mg once daily or 5 mg twice daily; maximum dose: 10 mg/day. Administration: Adult : May be administered without	Allergic symptoms, hay fever: General dosing: • Children ≥2 to <6 years: Oral: 5 mg once daily. • Children ≥6 years and Adolescents: 5 mg twice daily or 10 mg once daily. Product-specific dosing: • Children ≥2 to <6 years: Oral: Oral liquid or chewable tablet: 5 mg once daily. • Children ≥6 years	Altered kidney function: Oral: • CrCl ≥30 mL/minute: No dosage adjustment necessary • CrCl <30 mL/minute: 10 mg every 48 hours Dosing: Liver Impairment: Adult No dosage adjustment provided in manufacturer's labeling; however, hepatic impairment	Warnings/Precautions Disease-related concerns: • Hepatic impairment: Hepatic impairment increases systemic exposure. Use with caution. • Renal impairment: Use with caution in patients with renal impairment. Concurrent drug therapy issues: • Sedatives: Effects may be potentiated when used with other

		regard to meals. Dispersible tablet: Place in mouth and allow to dissolve. Swallow with or without water. Dosing: Older Adult Refer to adult dosing.	and Adolescents: Oral: - Oral liquid, capsule, tablet, or chewable tablet: 10 mg once daily. - Dispersible tablet: 5 mg twice daily or 10 mg once daily. Administration: Pediatric • Oral: Administer without regard to meals. • Rapidly disintegrating tablet: Place rapidly disintegrating tablet on the tongue; tablet disintegration occurs rapidly; may administer with or without water.	increases systemic exposure to loratadine.	sedative drugs or ethanol. Warnings: Additional Pediatric Considerations Safety and efficacy for the use of cough and cold products in pediatric patients <4 years of age is limited; the AAP warns against the use of these products for respiratory illnesses in young children. Serious adverse effects including death have been reported. Many of these products contain multiple active ingredients, increasing the risk of accidental overdose when used with other products. The FDA does not recommend OTC uses for these products in pediatric patients <2 years of age and recommends to use with caution in pediatric patients ≥2 years of age. Health care providers are reminded to ask caregivers about the use of OTC cough and cold products in order to avoid exposure to multiple medications containing the same ingredient (AAP 2018; CDC 2007; FDA 2017; FDA 2018).
Fexofenadine	Use: Labeled Indications Upper respiratory allergies: Temporary relief of runny nose, sneezing, itching of the nose or throat, and/or itchy, watery eyes due to hay fever or other upper respiratory allergies.	Upper respiratory allergies: Twice-daily formulations: Oral: 60 mg every 12 hours (maximum: 120 mg/day). Once-daily formulations: Oral: 180 mg once daily (maximum: 180 mg/day). Administration: Adult • Orally disintegrating tablet: Administer on an empty stomach. Remove tablet from individual blister and place immediately on tongue; tablet will disintegrate with or without water (do not administer with fruit juices). • Oral suspension, tablet: Administer	Allergic symptoms/rhinitis: Product-specific dosing: Twice-daily formulations (eg, oral suspension, oral disintegrating tablet, regular tablet): Oral suspension: -Infants ≥6 months and Children <2 years: Limited data available ; • <10.5 kg: Oral: 15 mg every 12 hours. • ≥10.5 kg: Oral: 30 mg every 12 hours. - Children ≥2 to <12 years: Oral: 30 mg every 12 hours; maximum daily dose: 60 mg/day. - Children ≥12 years and Adolescents: Oral: 60 mg every 12 hours; maximum daily dose: 120	Note : Use of the once-daily formulation is not recommended in patients with eGFR <50 mL/minute/1.73 m2 or on renal replacement therapies Altered kidney function : • eGFR ≥50 mL/minute/1.73 m2: Oral: No dosage adjustment necessary. • eGFR 10 to <50 mL/minute/1.73 m2: Oral: 60 mg every 12 to 24 hours. • eGFR <10 mL/minute/1.73 m2: Oral: 60 mg every 24 hours. Note: Results of a pharmacokinetic	Warnings/Precautions Disease-related concerns: • Renal impairment: Use with caution in patients with renal impairment; dosage adjustment may be recommended. Dosage form specific issues: • Orally disintegrating tablet: Some products may contain phenylalanine. Other warnings/precautions: • OTC labeling: When used for self-medication (OTC), do not exceed recommended dosage or administer at the same time with aluminum or

		with water only; do not administer with fruit juices. Shake suspension well before use. Use suspension only with enclosed dosing cup. Dosing: Older Adult Refer to adult dosing.	mg/day. Orally-disintegrating tablet (ODT): - Children ≥6 years to <12 years: Oral: 30 mg every 12 hours; maximum daily dose: 60 mg/day. - Children ≥12 years and Adolescents: Oral: 60 mg every 12 hours; maximum daily dose: 120 mg/day. 12-hour tablet: Children ≥12 years and Adolescents: Oral: 60 mg every 12 hours; maximum daily dose: 120 mg/day. Once-daily formulation: Children ≥12 years and Adolescents: Oral: 180 mg once daily. Administration: Pediatric Oral: Twice-daily formulations: • Suspension, regular tablet: May administer without respect to food. Take with water; avoid administration with fruit juices; shake suspension well before use. Administer suspension with an accurate measuring device; do not use a household teaspoon. • Orally-disintegrating tablet: Administer on an empty stomach. Do not remove from blister pack until ready to administer. Using dry hands, place immediately on tongue. Tablet will dissolve within seconds and may be swallowed with or without liquid; avoid administering with fruit juices. Once-daily formulation: Swallow whole with water; do not take with fruit juices.	study suggest that nonrenal clearance of fexofenadine is impaired in patients with end-stage kidney disease (ESKD); monitor patients for signs and symptoms of drug accumulation and toxicity. Dosing: Liver Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling.	magnesium antacids or with fruit juices. Warnings: Additional Pediatric Considerations Safety and efficacy for the use of cough and cold products in pediatric patients <4 years of age is limited; the AAP warns against the use of these products for respiratory illnesses in young children. Serious adverse effects including death have been reported. Many of these products contain multiple active ingredients, increasing the risk of accidental overdose when used with other products. The FDA does not recommend OTC uses for these products in pediatric patients <2 years of age and recommends to use with caution in pediatric patients ≥2 years of age. Health care providers are reminded to ask caregivers about the use of OTC cough and cold products in order to avoid exposure to multiple medications containing the same ingredient (AAP 2018; CDC 2007; FDA 2017; FDA 2018).
Levocetirizine	Use: Labeled Indications Allergic rhinitis (OTC only): Temporary relief of symptoms due to hay fever or other	Allergic rhinitis (OTC only): Oral: 5 mg once daily (in the evening); some patients with less severe symptoms may experience relief	Allergic rhinitis, perennial: Note: In children ≥2 years, dosing is based on previous FDA-approved manufacturer labeling	Dosing: Kidney Impairment: Adult Note: Renally adjusted dose recommendations are based on a usual dose of 5 mg/day.	Contraindications Known hypersensitivity to levocetirizine, cetirizine, or any component of the formulation;

	respiratory allergies (including rhinitis, sneezing, itchy/watery eyes, or itching of the throat/nose) in adults and pediatric patients ≥2 years of age. Allergic rhinitis, perennial: Relief of symptoms associated with perennial allergic rhinitis in pediatric patients 6 months to 2 years of age.	with 2.5 mg once daily; maximum dose: 5 mg/day. Dosing: Older Adult Refer to adult dosing; dosing should begin at the lower end of the dosing range. Administration: Adult Oral: Administer in the evening with or without food.	• Infants ≥6 months and Children ≤5 years: Oral: 1.25 mg once daily (in the evening). • Children 6 to 11 years: Oral: 2.5 mg once daily (in the evening). • Children ≥12 years and Adolescents: Oral: 5 mg once daily (in the evening); some patients may experience relief of symptoms with 2.5 mg once daily. Hay fever and other respiratory allergies: OTC labeling: • Children 2 to 5 years: Oral: 1.25 mg once daily (in the evening). • Children 6 to 11 years: Oral: 2.5 mg once daily (in the evening). • Children ≥12 years and Adolescents: Oral: 2.5 to 5 mg once daily (in the evening); dose based on symptom severity Administration: Pediatric Oral: Administer without regard to food in the evening.	• CrCl >80 mL/minute: No dosage adjustment necessary. • CrCl 50 to 80 mL/minute: 2.5 mg once daily. • CrCl 30 to 50 mL/minute: 2.5 mg once every other day. • CrCl 10 to 30 mL/minute: 2.5 mg twice weekly (every 3 or 4 days). • CrCl <10 mL/minute: Use is contraindicated. Dosing: Liver Impairment: Adult No dosage adjustment necessary.	end-stage renal disease (CrCl <10 mL/minute); hemodialysis; infants and children 6 months to 11 years of age with renal impairment Warnings/Precautions • CNS depression: May cause CNS depression, which may impair physical or mental abilities; patients must be cautioned about performing tasks which require mental alertness (eg, operating machinery or driving). • Pruritus: Pruritus, sometimes severe, has been reported within several days after stopping levocetirizine, usually after long-term (eg, months to years) use; some cases occurred in patients who did not have pruritus prior to therapy. If pruritus occurs after discontinuation, symptoms may improve by restarting or tapering levocetirizine. Disease-related concerns: • Renal impairment: Levocetirizine is excreted primarily by the kidneys; use with caution in adults with mild to severe renal impairment; dosage adjustments may be needed. • Urinary retention: Urinary retention may occur; use with caution in patients with increased risk of urinary retention (including spinal cord lesions or prostatic hyperplasia); discontinue if urinary retention occurs. Special populations: • Older adult: Use with caution in older adults.
Desloratadine	Use: Labeled Indications Allergic rhinitis: Relief of nasal and non-nasal symptoms of seasonal allergic rhinitis in adults and pediatric patients ≥2 years of age and	Allergic rhinitis, perennial or seasonal: Oral: 5 mg once daily. Administration: Adult Oral: May be taken with or without food.	Allergic rhinitis, perennial: • Infants 6 to ≤11 months: Oral: 1 mg once daily. • Children ≤5 years: Oral: 1.25 mg once daily. • Children 6 to ≤11 years: Oral: 2.5 mg	Dosing: Kidney Impairment: Adult Moderate to severe impairment: No dosage adjustment necessary; use with caution in severe impairment. Dosing: Liver	Warnings/Precautions Concerns related to adverse effects: • Hypersensitivity: Hypersensitivity reactions (including anaphylaxis) have been reported with use; discontinue

	perennial allergic rhinitis in adults and pediatric patients ≥ 6 months of age.	Dosing: Older Adult Refer to adult dosing.	once daily. • Children ≥ 12 years and Adolescents: Oral: 5 mg once daily. Allergic rhinitis, seasonal: • Children 2 to ≤ 5 years: Oral: 1.25 mg once daily. • Children 6 to ≤ 11 years: Oral: 2.5 mg once daily. • Children ≥ 12 years and Adolescents: Oral: 5 mg once daily. Administration: Pediatric Oral: May administer without regard to food.	Impairment: Adult No dosage adjustment necessary; use with caution in severe impairment.	therapy immediately with signs/symptoms of hypersensitivity. Disease-related concerns: • Hepatic impairment: Use with caution in patients with severe hepatic impairment. • Renal impairment: Use with caution in patients with severe renal impairment. Concurrent drug therapy issues: • Sedatives: Effects may be potentiated when used with other sedative drugs or ethanol. Special populations: • Slow metabolizers: Use with caution in patients known to be slow metabolizers of desloratadine (incidence of side effects may be increased). Warnings: Additional Pediatric Considerations • Desloratadine may increase susceptibility to seizures in pediatric patients, particularly in patients with a history of seizure disorders; 1 case series described 4 patients (ages: 5 to 16 years) who experienced seizures after initiation of desloratadine. Three of the 4 patients had known seizure disorders that were controlled prior to treatment with desloratadine. The final patient, who developed absence seizures after initiation of desloratadine, had a family history of febrile seizures but no personal history of seizure disorder (Cerminara 2013). • Safety and efficacy for the use of cough and cold products in pediatric patients < 4 years of age are limited; the AAP warns against the use of these products for respiratory illnesses in young children. Serious adverse effects including death have been reported. Many of these products contain multiple
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					active ingredients, increasing the risk of accidental overdose when used with other products. Health care providers are reminded to ask caregivers about the use of OTC cough and cold products in order to avoid exposure to multiple medications containing the same ingredient (AAP 2018; CDC 2007; FDA 2017; FDA 2018).
Bilastine	Use: Labeled Indications Note: Not approved in the United States. Allergic rhinitis: Relief of symptoms associated with seasonal allergic rhinitis in patients ≥4 years of age and weighing ≥16 kg.	Allergic rhinitis: Oral: 20 mg once daily (maximum: 20 mg/day). Administration: Adult Oral: Administer with water 1 hour before or 2 hours after intake of food or fruit juices Dosing: Older Adult Refer to adult dosing.	Allergic rhinitis, seasonal: • Children ≥4 to 11 years weighing ≥16 kg: Oral: 10 mg once daily. • Children ≥12 years and Adolescents: Oral: 20 mg once daily. Administration: Pediatric • Oral: Administer with water 1 hour before or 2 hours after intake of food or fruit juices. • Oral dispersible tablet: Place on tongue to allow tablet to dissolve so it can be easily swallowed. May also disperse in water prior to administration. • Oral solution: Administer with an accurate measuring device (dosing cup provided); do not use a household teaspoon (overdosage may occur).	Dosing: Kidney Impairment: Adult No dosage adjustment necessary. Dosing: Liver Impairment: Adult No dosage adjustment necessary.	Contraindications Hypersensitivity to bilastine or any component of the formulation; history of QT prolongation and/or torsades de pointes, including congenital long QT syndromes. Warnings/Precautions Disease-related concerns: • QT interval prolongation: QTc interval prolongation has been reported with use; use is contraindicated in patients with a history of QT prolongation and/or Torsade de pointes, including congenital long QT syndromes. Use caution in patients with a history of cardiac arrhythmia, significant bradycardia, a family history of sudden cardiac death, electrolyte abnormalities (eg, hypokalemia, hypomagnesemia), or with concomitant use of other QTc-prolonging drugs.

Drugs for Common cold

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่ /Dose เด็ก	Dose ปรับตามตัว/ไต	ข้อห้ามใช้/ควรระวัง
Decolgen prin /TIFFY DEY (Paracetamol 500 มิลลิกรัม + Chlorpheniramine maleate 2 มิลลิกรัม + Phenylephrine HCl	ดีคอลเจน พรีน (Decolgen prin) / ทิฟฟี่ เดย์ (TIFFY DEY) มีสรรพคุณช่วย บรรเทาอาการคัดจมูก น้ำมูกไหล น้ำตาไหล คันคอ จาม ปวดศีรษะ	ดีคอลเจน พรีน (Decolgen prin) / ทิฟฟี่ เดย์ (TIFFY DEY) เป็นยาที่เหมาะสมกับอาการไข้หวัดที่มี อาการแพ้ เช่น ไอ จาม และมีน้ำมูกมาก สำหรับยา เม็ด ผู้ใหญ่ ให้รับประทานยาครั้งละ 1-2 เม็ด ทุก 4-6 ชั่วโมง เด็กอายุ 6-12 ปี ให้รับประทานยาครั้งละ 1 เม็ด ทุก 4-6 ชั่วโมง สำหรับยาน้ำเชื่อมทิฟฟี่ ไซรัป (TIFFY SYRUP) ใน 1 ช้อนชา (5 มิลลิลิตร) จะ	ควรระมัดระวังการใช้ ในผู้ที่มีการแพ้ยาหรือไต ทำงานผิดปกติ	ข้อห้าม/ข้อควรระวัง ในการใช้ยาดีคอลเจน/ ทิฟฟี่ • ห้ามใช้ยาในผู้ที่มี ประวัติการแพ้ ส่วนประกอบหรือด้วยยา ไตในยานี้

10 มิลลิกรัม) ส่วน (TIFFY SYRUP) ใน 1 ขอนชา (5 มิลลิลิตร) จะประกอบไปด้วยยาพาราเซตามอล 120 มิลลิกรัม + คลอร์เฟนิรามีน 1 มิลลิกรัม + ฟีนีลเอฟรีน 5 มิลลิกรัม)	และเป็นใช้ อันเนื่องมาจากโรคภูมิแพ้ โรคหัด หรือไข้ละอองฟาง กลไกการออกฤทธิ์ของยาดีคอลเจน/ทิฟฟี ด้วยยาพาราเซตามอล (Paracetamol) เป็นยาแก้ปวดลดไข้ มีสรรพคุณช่วยบรรเทาอาการไข้ (ยาลดไข้) และบรรเทาอาการปวด (ยาแก้ปวด) ด้วยยาคลอร์เฟนิรามีน มาลีเอต (Chlorpheniramine maleate) เป็นยาแก้แพ้ มีสรรพคุณช่วยลดอาการแพ้ต่าง ๆ เช่น ไอ จาม น้ำมูกไหล คัดจมูก จากโรคหัด ด้วยยาฟีนีลเอฟรีน ไฮโดรคลอไรด์ (Phenylephrine HCl) เป็นยาลดน้ำมูก มีสรรพคุณช่วยลดปริมาณน้ำมูกที่เป็นเหตุทำให้คัดจมูกและหายใจไม่สะดวก ด้วยยาซูโดอีเฟดรีน ไฮโดรคลอไรด์ (Pseudoephedrine HCl) เป็นยาบรรเทาอาการคัดจมูกจากหัด ไข้หัดใหญ่ ไข้ละอองฟาง และโรคภูมิแพ้ทางเดินหายใจอื่น ๆ ด้วยกลไกของยาดังกล่าวจึงทำให้ยาดีคอลเจน / ทิฟฟี เกิดฤทธิ์การรักษาตามสรรพคุณที่กล่าวมา ซึ่งหลังจากรับประทานยาเข้าไปแล้วตัวยาจะเริ่มออกฤทธิ์ภายในเวลาประมาณ 15-30 นาที และจะใช้เวลาออกฤทธิ์ในร่างกายประมาณ 4-6 ชั่วโมง จึงหมดฤทธิ์ เพราะจะค่อย ๆ ถูกร่างกายกำจัดออกไปทางปัสสาวะ ทางเหงื่อ ทางอุจจาระ ฯลฯ ดังนั้นเมื่อยาหมดฤทธิ์แล้วอาการของโรคอาจจะกลับมาเป็นอีก ก็ต้องรับประทานยาซ้ำทุก 4-6 ชั่วโมง Ref : https://medthai.com/%e0%b8%94%e0%b8%b5%e0%b8%84%e0%b8%ad%e0%b8%a5%e0%b9%80%e0%b8%88%e0%b8%99-%e0%b8%97%e0%b8%b4%e0%b8%9f%e0%b8%9f%e0%b8%b5%e0%b9%88/ Medthai	ประกอบไปด้วยยาพาราเซตามอล 120 มิลลิกรัม + คลอร์เฟนิรามีน 1 มิลลิกรัม + ฟีนีลเอฟรีน 5 มิลลิกรัม ผู้ใหญ่ ให้รับประทานยาครั้งละ 2 ขอนชา ทุก 4-6 ชั่วโมง เด็กอายุ 6-12 ปี ให้รับประทานยาครั้งละ 1-2 ขอนชา ทุก 4-6 ชั่วโมง เด็กอายุ 3-6 ปี ให้รับประทานยาครั้งละ 1 ขอนชา ทุก 4-6 ชั่วโมง เด็กอายุ 1-3 ปี ให้รับประทานยาครั้งละ ½-1 ขอนชา ทุก 4-6 ชั่วโมง	• ห้ามใช้ยานี้ร่วมกับยากลุ่ม Monoamine oxidase inhibitors (MAOIs) สำหรับยาดีคอลเจน ฟรีน (Decolgen prin) / ทิฟฟี เดย์ (TIFFY DEY) ที่มีตัวยาฟีนีลเอฟรีน (Phenylephrine) เป็นส่วนประกอบ • ห้ามใช้กับผู้ที่มีปวดยโรคความดันโลหิตสูง โรคเบาหวาน โรคหัวใจ โรคหลอดเลือด โรคไต หรือโรคของต่อมไทรอยด์ นอกจากแพทย์สั่ง (สำหรับผู้ป่วยเบาหวาน ยาฟีนีลเอฟรีนอาจทำให้เสี่ยงต่อการเกิดโรคหัวใจและหลอดเลือดเพิ่มขึ้น แต่ไม่ได้ทำให้น้ำตาลในเลือดเพิ่มขึ้นแต่อย่างใด) • ห้ามใช้ยานี้ในเด็กอายุต่ำกว่า 1 ปี ผู้ที่กำลังมีอาการหอบหืด โรคต่อหีน (ยานี้อาจทำให้ความดันภายในตาเพิ่มขึ้น) ต่อมลูกหมากโต หรือปัสสาวะขัด นอกจากแพทย์สั่ง • ห้ามใช้ยาทั้งหมดอายุหรือเสื่อมสภาพแล้ว • ไม่ควรใช้ยานี้ติดต่อกันเกิน 5 วัน หรือเกินกว่าขนาดที่กำหนด เพราะจะทำให้เป็นพิษต่อตับไต (ผู้ป่วยโรคตับ โรคไต ควรปรึกษาแพทย์หรือเภสัชกรก่อนใช้ยานี้) • หลีกเลี่ยงการรับประทานยาร่วมกับการดื่มเครื่องดื่มที่มีแอลกอฮอล์เป็นส่วนประกอบ • ควรระมัดระวังการใช้ยานี้ร่วมกับยาที่มีฤทธิ์กดประสาทส่วนกลาง เช่น ยานอนหลับ ยาต้านอาการซึมเศร้าชนิดต่าง ๆ ยาที่มีฤทธิ์แอนติโคลีเนอร์จิก • ควรระมัดระวังการใช้ยานี้กับผู้ป่วยโรคความดันโลหิตสูง โรคหัวใจ และหลอดเลือด โรคของต่อมไทรอยด์ ผู้ที่มีภาวะตับหรือไตทำงานผิดปกติ และผู้ที่มีประวัติเป็นโรคหอบหืด ภูมิแพ้ โรคจิตสับสนงมุก หรือโรคถุงลมโป่งพอง • ควรระมัดระวังการใช้ยาในหญิงตั้งครรภ์ (โดยเฉพาะหญิงตั้งครรภ์ระยะ 3 เดือนแรก) และหญิงให้นมบุตร ซึ่งการใช้ยาในผู้ป่วยกลุ่มนี้จะต้องเป็นไปตามคำสั่งของแพทย์เท่านั้น ผู้ป่วยห้ามหาซื้อยามารับประทานด้วยตัวเอง • ควรระมัดระวังการใช้ยานี้ในเด็กและผู้สูงอายุ โดยเฉพาะในเด็กอายุต่ำกว่า 7 ปี การใช้ยาจะต้องอยู่ในดุลยพินิจของแพทย์เท่านั้น (เด็กและ
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				ผู้สูงอายุจะไวต่อยานี้มากขึ้น ทำให้ง่วงนอน วิงเวียน ประสาทหลอน ความดันโลหิตต่ำ นอนไม่หลับ ปากแห้ง ปัสสาวะคั่ง และบางรายอาจมีอาการชักได้) • ยานี้อาจทำให้เกิดอาการง่วงนอน วิงเวียน สับสน ปากแห้ง ตาพร่า เสมหะเหนียวขึ้น ปัสสาวะขัด และเม็ดเลือดผิดปกติได้ • หลังการรับประทานยา จึงควรหลีกเลี่ยงหรือเพิ่มความระมัดระวังในการขับถ่ายยานพาหนะหรือทำงานเกี่ยวกับเครื่องจักร หรือทำงานที่เสี่ยงต่อการพลัดตกจากที่สูง
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Analgesics-Anti-inflammation-Antipyretic

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตับ/ไต	ข้อห้ามใช้/ควรระวัง																																																				
Paracetamol	Use: Labeled Indications Fever: Temporary reduction of fever. Pain: Oral, Rectal: Temporary relief of minor aches, pains, and headache.	Pain (mild to moderate) or fever: Oral: 325 to 650 mg every 4 to 6 hours as needed or 1 g every 6 hours as needed; maximum dose: 4 g/day (Ref). See "Note" above regarding maximum dose. OTC labeling (patient-guided therapy): Note: Dosage recommendations, including maximum doses, vary among OTC manufacturers. Immediate release: Regular strength (325 mg/tablet): 2 tablets (650 mg) every 4 to 6 hours as needed; maximum daily dose: 10 tablets/day (3.25 g/day). Extra strength (500 mg/tablet): 2 tablets (1 g) every 6 hours as needed; maximum daily dose: 6 tablets/day (3 g/day). Extended release (650 mg/tablet): 2 tablets (1.3 g) every 8 hours as needed; maximum daily dose: 6 tablets/day (3.9 g/day).	Pain (mild to moderate) or fever: Oral: Weight-directed dosing: Infants, Children, and Adolescents: 10 to 15 mg/kg/dose every 4 to 6 hours as needed; do not exceed 5 doses in 24 hours; maximum daily dose: 75 mg/kg/day not to exceed 4,000 mg/day Fixed dosing: Oral suspension, chewable tablets: Infants and Children <12 years: Consult specific product formulations for appropriate age groups. See table; use of weight to select dose is preferred; if weight is not available, then use age; doses may be repeated every 4 hours; maximum: 5 doses/day. <table border="1"><caption>Weight-based dosing for acetaminophen (mg/kg)</caption><thead><tr><th>Weight (kg)</th><th>Age</th><th>Dose</th><th>Frequency</th></tr></thead><tbody><tr><td>10-15</td><td>2-5 years</td><td>100 mg</td><td>Every 4-6 hours</td></tr><tr><td>16-20</td><td>6-11 years</td><td>150 mg</td><td>Every 4-6 hours</td></tr><tr><td>21-25</td><td>12-17 years</td><td>200 mg</td><td>Every 4-6 hours</td></tr><tr><td>26-30</td><td>18-24 years</td><td>250 mg</td><td>Every 4-6 hours</td></tr><tr><td>31-35</td><td>25-34 years</td><td>300 mg</td><td>Every 4-6 hours</td></tr><tr><td>36-40</td><td>35-44 years</td><td>350 mg</td><td>Every 4-6 hours</td></tr><tr><td>41-45</td><td>45-54 years</td><td>400 mg</td><td>Every 4-6 hours</td></tr><tr><td>46-50</td><td>55-64 years</td><td>450 mg</td><td>Every 4-6 hours</td></tr><tr><td>51-55</td><td>65-74 years</td><td>500 mg</td><td>Every 4-6 hours</td></tr><tr><td>56-60</td><td>75-84 years</td><td>550 mg</td><td>Every 4-6 hours</td></tr><tr><td>61-65</td><td>85-94 years</td><td>600 mg</td><td>Every 4-6 hours</td></tr><tr><td>66-70</td><td>95-104 years</td><td>650 mg</td><td>Every 4-6 hours</td></tr></tbody></table> Immediate-release solid dosage formulations: Note: Actual OTC dosing recommendations may vary by product and/or manufacturer:	Weight (kg)	Age	Dose	Frequency	10-15	2-5 years	100 mg	Every 4-6 hours	16-20	6-11 years	150 mg	Every 4-6 hours	21-25	12-17 years	200 mg	Every 4-6 hours	26-30	18-24 years	250 mg	Every 4-6 hours	31-35	25-34 years	300 mg	Every 4-6 hours	36-40	35-44 years	350 mg	Every 4-6 hours	41-45	45-54 years	400 mg	Every 4-6 hours	46-50	55-64 years	450 mg	Every 4-6 hours	51-55	65-74 years	500 mg	Every 4-6 hours	56-60	75-84 years	550 mg	Every 4-6 hours	61-65	85-94 years	600 mg	Every 4-6 hours	66-70	95-104 years	650 mg	Every 4-6 hours	Dosing: Kidney Impairment: Adult Altered kidney function: Mild to severe impairment: IV, Oral, Rectal: No dosage adjustment likely to be necessary. The manufacturer's labeling for IV acetaminophen states that longer dosing intervals and a reduced total daily dose may be warranted in patients with severe kidney impairment (CrCl ≤30 mL/minute); however, acetaminophen concentrations and half-life are increased but similar to those in patients with normal renal function. Glucuronide and sulfate conjugate metabolites accumulate in renal impairment, but the clinical effects are unknown Dosing: Kidney Impairment: Pediatric Altered kidney function: Infants, Children, and Adolescents: Oral, rectal: Mild to severe impairment: There are no dosage adjustments provided in the manufacturer's	Warnings/Precautions Disease-related concerns: • Hepatic impairment: Use with caution in patients with hepatic impairment or active liver disease; use of the IV formulation is contraindicated in patients with severe hepatic impairment or severe active liver disease. • Hypovolemia: Use the IV formulation with caution in patients with severe hypovolemia (eg, due to dehydration or blood loss). Warnings: Additional Pediatric Considerations Prophylactic use of acetaminophen to reduce fever and discomfort associated with vaccination is not recommended by the Advisory Committee on Immunization Practices (ACIP).
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			Children 6 to 11 years: 325 mg every 4 to 6 hours; maximum daily dose: 1,625 mg/day; Note: Do not use more than 5 days unless directed by a physician. Children ≥12 years and Adolescents: Regular strength: 650 mg every 4 to 6 hours; maximum daily dose: 3,250 mg/day unless directed by a physician; under physician supervision daily doses ≤4,000 mg may be used. Extra strength: 1,000 mg every 6 hours; maximum daily dose: 3,000 mg/day unless directed by a physician; under physician supervision daily doses ≤4,000 mg may be used. Extended release: Children ≥12 years and Adolescents: 1,300 mg every 8 hours; maximum daily dose: 3,900 mg/day.	labeling; based on adult pharmacokinetic studies, dosage adjustment may not be necessary for short courses. In adult pharmacokinetic studies, plasma concentrations of acetaminophen did not differ in renal impairment patients when compared to healthy patients for short courses of treatment (ie, 3 days); however, accumulation of the glucuronide and sulfate conjugate metabolites in renal impairment has been described following a single dose of acetaminophen up to repeat dosing for 10 days; the clinical significance of this finding is unknown	
Aspirin	Use: Labeled Indications Immediate release: Analgesic, antipyretic, and anti-inflammatory: For the temporary relief of headache, pain, and fever caused by colds, muscle aches and pains, menstrual pain, toothache pain, and minor aches and pains of arthritis.	Dosing: Adult Pain or fever: Immediate release: Oral: 325 mg to 1 g every 4 to 6 hours as needed; usual maximum daily dose: 4 g/day . Note: If patient cannot take orally, a rectal suppository (300 mg) is available.	Dosing: Pediatric Analgesic: Note: Do not use aspirin in pediatric patients <18 years who have or who are recovering from chickenpox or flu symptoms (eg, viral illness) due to the association with Reye syndrome. Rectal suppository may not be appropriate for all doses. Infants and Children <12 years: Limited data available: Oral, rectal: 10 to 15 mg/kg/dose (maximum dose: 650 mg/dose) every 4 to 6 hours as needed; maximum daily dose: 90 mg/kg/day or 3,900 mg/day,	Dosing: Kidney Impairment: Adult Analgesia or anti-inflammatory uses: CrCl >10 mL/minute: No dosage adjustment necessary; however, high doses have been associated with acute kidney injury (AKI) (Ref). Use lowest effective dosage and limit duration of therapy, particularly for patients at high risk for developing AKI (eg, patients with chronic kidney disease, volume depletion, older age) (Ref). CrCl <10 mL/minute: Avoid use (Ref). May exacerbate uremic GI and hematologic symptoms Dosing: Pediatric Analgesic: Note: Do not use aspirin in pediatric patients <18 years who have or who are recovering	Contraindications Hypersensitivity to NSAIDs: patients with asthma, rhinitis, and nasal polyps; use in children or teenagers for viral infections, with or without fever. Warnings/Precautions Disease-related concerns: • Bariatric surgery: Presurgical assessment of the indication for use, symptoms, and goals of therapy should be documented to enable postsurgical assessment. Aspirin use following bariatric surgery is controversial; while efficacy may improve, the risk of adverse effects may increase • Bleeding disorders: Use with caution in patients with platelet and bleeding disorders.

			whichever is less . Children ≥12 years and Adolescents: Oral, rectal: 325 to 650 mg every 4 to 6 hours as needed; maximum daily dose: 3,900 mg/day	from chickenpox or flu symptoms (eg, viral illness) due to the association with Reye syndrome. Rectal suppository may not be appropriate for all doses. Infants and Children <12 years: Limited data available: Oral, rectal: 10 to 15 mg/kg/dose (maximum dose: 650 mg/dose) every 4 to 6 hours as needed; maximum daily dose: 90 mg/kg/day or 3,900 mg/day, whichever is less . Children ≥12 years and Adolescents: Oral, rectal: 325 to 650 mg every 4 to 6 hours as needed; maximum daily dose: 3,900 mg/day .Anti-inflammatory: Limited data available: Infants, Children, and Adolescents: Oral: Initial: 60 to 90 mg/kg/day in divided doses; usual maintenance: 80 to 100 mg/kg/day divided every 6 to 8 hours; monitor serum concentrations (Ref). Note: Although included in some product labeling, the use of salicylates including aspirin for the treatment of juvenile idiopathic arthritis has been replaced by other pharmacologic agents with improved efficacy and adverse effect profiles (eg, nonsalicylate nonsteroidal anti-inflammatory drugs, disease-modifying antirheumatic drugs, glucocorticoids, biologic modifiers)	• Dehydration: Use with caution in patients with dehydration • Ethanol use: Heavy ethanol use (>3 drinks/day) can increase bleeding risks • Gastrointestinal disease: Use with caution in patients with erosive gastritis. Avoid use in patients with active peptic ulcer disease. • Hepatic impairment: Avoid use in severe hepatic failure. • Renal impairment: Use high dosages impairment Warnings: Additional Pediatric Considerations Do not use aspirin in pediatric patients <18 years of age who have or who are recovering from chickenpox or flu symptoms when using aspirin, changes in behavior) may be an early sign of Reye syndrome; instruct patients and caregivers to contact their health care provider if these symptoms occur; patients should be kept current on their influenza and varicella immunizations. Although Reye syndrome has been observed in patients receiving prolonged, high-dose aspirin therapy after Kawasaki disease presentation; it has not been observed in pediatric patients receiving low (antiplatelet) dosing regimens. Patients with Kawasaki disease and presenting with influenza or viral illness should not receive aspirin; acetaminophen is suggested as an antipyretic in these patients, and an alternate antiplatelet agent is suggested for a minimum of 2 weeks
Ibuprofen	Use: Labeled Indications Oral:	Dosing: Adult Anti-inflammatory	Dosing: Pediatric -Pain	Dosing: Kidney Impairment: Adult	Contraindications Hypersensitivity to

	Management of inflammatory diseases and rheumatoid disorders, mild to moderate pain, fever, dysmenorrhea, and osteoarthritis OTC labeling: Reduction of fever; management of pain due to headache, acute migraine, sore throat, arthritis, or physical or athletic overexertion (eg, sprains/strains), menstrual pain, dental pain, minor muscle/bone/joint pain, backache, and pain due to the common cold and flu.	(eg, for arthritis associated with rheumatic disease): Oral: 400 to 800 mg every 6 to 8 hours; maximum dose: 3.2 g/day. Some experts generally recommend a maximum dose of 2.4 g/day for chronic use, except during a disease flare when up to 3.2 g/day may be considered for several weeks until flare resolves . Dysmenorrhea Oral: Initial: 400 mg every 4 hours as needed or 600 to 800 mg every 6 to 8 hours as needed; maximum dose: 3.2 g/day. Begin at menses onset or 1 to 2 days prior to onset of menses for severe symptoms; usual duration: 1 to 5 days . Fever (alternative agent): IV, Oral: 200 to 400 mg every 4 to 6 hours as needed; if fever persists, may titrate up to 600 to 800 mg every 6 hours as needed; maximum dose: 3.2 g/day . OTC labeling (patient-guided therapy): Oral: 200 mg every 4 to 6 hours as needed; if no relief, may increase to 400 mg every 4 to 6 hours as needed; maximum dose: 1.2 g/day. Use for >3 days is not recommended unless directed by health care provider. Gout, treatment, acute flares (off-label use): Oral: Initial: 800 mg every 8 hours within 24 to 48 hours of flare onset; reduce dose as symptoms improve. Discontinue a few days after resolution of clinical signs (usual total duration: 5 to 7 days); longer duration may be required if therapy is	Oral: Weight-directed dosing: Infants, Children, and Adolescents: Limited data available in infants <6 months: Oral: 4 to 10 mg/kg/dose (maximum dose: 600 mg/dose) every 6 to 8 hours; maximum daily dose: 40 mg/kg/day or 2,400 mg/day, whichever is less (Ref). Fixed dosing: Infants ≥6 months and Children ≤11 years: Oral: See table based upon manufacturer's labeling; use of weight to select dose is preferred; if weight is not available, then use age; doses may be repeated every 6 to 8 hours; maximum: 4 doses/day. <table><tr><th>Weight (kg)</th><th>Weight (lb)</th><th>Dose (mg)</th><th>Dose (mg/kg)</th></tr><tr><td>10</td><td>22</td><td>40</td><td>4</td></tr><tr><td>15</td><td>33</td><td>60</td><td>4</td></tr><tr><td>20</td><td>44</td><td>80</td><td>4</td></tr><tr><td>25</td><td>55</td><td>100</td><td>4</td></tr><tr><td>30</td><td>66</td><td>120</td><td>4</td></tr><tr><td>35</td><td>77</td><td>140</td><td>4</td></tr><tr><td>40</td><td>88</td><td>160</td><td>4</td></tr><tr><td>45</td><td>99</td><td>180</td><td>4</td></tr><tr><td>50</td><td>110</td><td>200</td><td>4</td></tr><tr><td>55</td><td>121</td><td>220</td><td>4</td></tr><tr><td>60</td><td>132</td><td>240</td><td>4</td></tr><tr><td>65</td><td>143</td><td>260</td><td>4</td></tr><tr><td>70</td><td>154</td><td>280</td><td>4</td></tr><tr><td>75</td><td>165</td><td>300</td><td>4</td></tr><tr><td>80</td><td>176</td><td>320</td><td>4</td></tr><tr><td>85</td><td>187</td><td>340</td><td>4</td></tr><tr><td>90</td><td>198</td><td>360</td><td>4</td></tr><tr><td>95</td><td>209</td><td>380</td><td>4</td></tr><tr><td>100</td><td>220</td><td>400</td><td>4</td></tr></table> -Fever. Oral: Weight-directed dosing: Infants ≥3 months weighing ≥5 kg, Children, and Adolescents: Limited data available in infants <6 months: Oral: 5 to 10 mg/kg/dose (maximum dose: 600 mg/dose) every 6 to 8 hours; maximum daily dose: 40 mg/kg/day or 2,400 mg/day, whichever is less (Ref). Fixed dosing: Note: Treatment for >3 days is not recommended unless directed by health care provider. Infants ≥6 months and Children ≤11 years: Oral: See table based upon manufacturer's labeling; use of weight to select dose is preferred; if weight is not available, then use age; doses may be repeated every 6 to 8 hours; maximum: 4 doses/day.	Weight (kg)	Weight (lb)	Dose (mg)	Dose (mg/kg)	10	22	40	4	15	33	60	4	20	44	80	4	25	55	100	4	30	66	120	4	35	77	140	4	40	88	160	4	45	99	180	4	50	110	200	4	55	121	220	4	60	132	240	4	65	143	260	4	70	154	280	4	75	165	300	4	80	176	320	4	85	187	340	4	90	198	360	4	95	209	380	4	100	220	400	4	Altered kidney function: IV, Oral: CrCl ≥60 mL/minute: No dosage adjustment necessary (Ref). CrCl >30 to <60 mL/minute: No dosage adjustment necessary (Ref). Use of analgesics other than nonsteroidal anti-inflammatory drugs may be preferred. If necessary, use the lowest effective dose for the shortest duration possible; avoid in patients at high risk for acute kidney injury (ie, volume depleted, hypotensive, elderly, or taking concurrent nephrotoxic medications) (Ref). CrCl ≤ 30 mL/minute: Avoid use due to increased risk of acute kidney injury (Ref). Dosing: Kidney Impairment: Pediatric Infants, Children, and Adolescents: Oral, IV (Caldolor): There are no dosage adjustments provided in the manufacturer's labeling; avoid use in advanced disease. KDIGO guidelines provide the following recommendations for NSAIDs (Ref): eGFR 30 to <60 mL/minute/1.73 m2: Avoid use in patients with intercurrent disease that increases risk of acute kidney injury. eGFR <30 mL/minute/1.73 m2: Avoid use.	ibuprofen (eg, anaphylactic reactions, serious skin reactions) or any component of the formulation; history of asthma, urticaria, or allergic-type reaction to aspirin or other NSAIDs; aspirin triad (eg, bronchial asthma, aspirin intolerance, rhinitis); use in the setting of coronary artery bypass graft (CABG) surgery Warnings/Precautions Disease-related concerns: • Aseptic meningitis: May increase the risk of aseptic meningitis, especially in patients with systemic lupus erythematosus and mixed connective tissue disorders. • Asthma: Contraindicated in patients with aspirin-sensitive asthma; severe and potentially fatal bronchospasm may occur. Use caution in patients with other forms of asthma. • Bariatric surgery: Gastric ulceration: Avoid chronic use of oral nonselective NSAIDs after bariatric surgery; development of anastomotic ulcerations/perforations may occur • Hepatic impairment: Use with caution in patients with hepatic impairment.
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		delayed . Migraine, acute treatment Note: Limit use to ≤14 days per month to avoid medication-overuse headache . For use as monotherapy in mild to moderate attacks not associated with vomiting or severe nausea; may be used in combination with triptans for severe migraine . Administration early in the course of a migraine attack, at the first sign of pain, may improve response to treatment . Oral: 400 to 600 mg once . <i>OTC labeling (patient-guided therapy):</i> Oral: 400 mg at onset of symptoms. Pain IV, Oral: 200 to 400 mg every 4 to 6 hours as needed or 600 to 800 mg every 6 to 8 hours as needed; maximum dose: 3.2 g/day . For postoperative pain, doses may be scheduled initially . Some experts suggest a usual maximum of 2.4 g/day for chronic use due to increased adverse effects with higher doses . <i>OTC labeling (patient-guided therapy):</i> Oral: 200 mg every 4 to 6 hours as needed; if no relief, may increase to 400 mg every 4 to 6 hours as needed; maximum dose: 1.2 g/day. Use for >10 days is not recommended unless directed by health care provider.	<table><tr><th colspan="4">Pharmacokinetics</th></tr><tr><th>Parameter</th><th>Value</th><th>Range</th><th>Notes</th></tr><tr><td>Half-life (t_{1/2})</td><td>2.0 h</td><td>1.5-2.5 h</td><td></td></tr><tr><td>Volume of distribution (V_d)</td><td>0.15 L/kg</td><td>0.1-0.2 L/kg</td><td></td></tr><tr><td>Clearance (CL)</td><td>0.15 L/kg/h</td><td>0.1-0.2 L/kg/h</td><td></td></tr><tr><td>Protein binding</td><td>99%</td><td>98-99%</td><td></td></tr><tr><td>Metabolism</td><td>99%</td><td>98-99%</td><td></td></tr><tr><td>Excretion</td><td>1%</td><td>0.5-1.5%</td><td></td></tr></table>	Pharmacokinetics				Parameter	Value	Range	Notes	Half-life (t _{1/2})	2.0 h	1.5-2.5 h		Volume of distribution (V _d)	0.15 L/kg	0.1-0.2 L/kg		Clearance (CL)	0.15 L/kg/h	0.1-0.2 L/kg/h		Protein binding	99%	98-99%		Metabolism	99%	98-99%		Excretion	1%	0.5-1.5%			
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Diclofenac	Dysmenorrhea: Treatment of primary dysmenorrhea. Migraine, acute	Dosing: Adult	Dosing: Pediatric Acute pain Acute pain (mild to	Dosing: Kidney Impairment: Adult Altered kidney	Contraindications Hypersensitivity to diclofenac (eg, anaphylactic																																

	treatment: Acute treatment of migraine attacks with or without aura in adults. Osteoarthritis: Relief of signs and symptoms of osteoarthritis. Pain, acute: Relief of mild to moderate acute pain in pediatric patients ≥ 12 years of age (Zipsor) and adults. Rheumatoid arthritis: Relief of signs and symptoms of rheumatoid arthritis.	Acute pain Oral: <i>Diclofenac potassium or sodium formulations</i>: 100 to 150 mg/day in divided doses as needed or scheduled (may choose immediate release [given in 2 to 4 divided doses], delayed release [given in 2 to 4 divided doses], or extended release [given once daily]); may administer 100 mg as an initial loading dose followed by a maintenance dose; maximum dose (after day 1): 150 mg/day . Oral: <i>Diclofenac acid</i>: 18 mg or 35 mg 3 times daily as needed or scheduled; maximum dose: 105 mg/day. Ankylosing spondylitis: Oral: <i>Diclofenac potassium or sodium formulations</i>: 100 to 150 mg/day in divided doses (may choose immediate release [given in 2 to 4 divided doses], delayed release [given in 2 to 4 divided doses], or extended release [given once daily]); maximum dose: 150 mg/day . Dysmenorrhea: Oral: <i>Diclofenac potassium or sodium formulations</i>: 150 mg/day in divided doses (may choose immediate release [given in 2 to 4 divided doses] or delayed release [given in 2 to 4 divided doses]); may administer 75 to 100 mg as an initial loading dose followed by a maintenance dose; maximum dose (after day 1): 150 mg/day. Begin at menses onset or 1 to 2 days prior to onset of menses for severe symptoms; usual duration: 1 to 5 days Gout, treatment, acute flares	moderate): Children ≥ 12 years and Adolescents: Immediate-release capsule (Zipsor): Oral: 25 mg 4 times daily . Juvenile idiopathic arthritis Limited data available: Children and Adolescents: Immediate-release tablet: Oral: 2 to 3 mg/kg/day in divided doses 2 to 3 times daily; maximum daily dose: 150 mg/day . Migraine Limited data available: Children ≥ 12 years and Adolescents: Powder for oral solution (eg, Cambia): Oral: 50 mg (1 packet) as a single dose at onset of migraine . Juvenile idiopathic arthritis Limited data available: Children: Suppository (12.5 mg or 25 mg only): Rectal: 1 to 3 mg/kg/day in divided doses 2 to 3 times daily . Osteoarthritis Limited data available: Adolescents ≥ 16 years: Suppository (50 mg or 100 mg): Rectal: Insert 50 or 100 mg suppository at bedtime as a substitute for the last daily oral dose; maximum daily dose: 100 mg/day (oral and rectal) . Postoperative pain Limited data available: Rectal suppository (12.5, 25, or 50 mg): Children and Adolescents: Suppository (12.5 mg, 25 mg, or 50 mg): Rectal: Initial dose: 1 to 2 mg/kg/dose once followed by 1 mg/kg/dose 3 times daily; round dose to nearest available suppository size; maximum single	function: Oral, rectal: CrCl ≥ 60 mL/minute: No dosage adjustment necessary CrCl >30 to <60 mL/minute: No dosage adjustment necessary . However, use of analgesics other than nonsteroidal anti-inflammatory drugs may be preferred. If necessary, use the lowest effective dose for the shortest duration possible; avoid in patients at high risk for acute kidney injury. CrCl ≤ 30 mL/minute: Avoid use due to increased risk of acute kidney injury Dosing: Kidney Impairment: Pediatric eGFR 30 to <60 mL/minute/1.73 m²: Temporarily discontinue in patients with intercurrent disease that increases risk of acute kidney injury eGFR <30 mL/minute/1.73 m²: Avoid use.	reactions, serious skin reactions) or bovine protein (Zipsor only) or any component of the formulation; history of asthma, urticaria, or other allergic-type reactions after taking aspirin or other nonsteroidal anti-inflammatory drugs; use in the setting of coronary artery bypass graft surgery. Warnings/Precautions <i>Concerns related to adverse effects:</i> -Anaphylactoid reactions -Cardiovascular events -CNS effects -GI events -Hepatic effects -Renal effects -Hyperkalemia -Skin reactions/hypersensitivity
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		(off-label use): Oral: <i>Diclofenac potassium or sodium formulations</i>: 100 to 150 mg/day in divided doses (may choose immediate release [given in 2 to 4 divided doses], delayed release [given in 2 to 4 divided doses], or extended release [given once daily]). Initiate within 24 to 48 hours of flare onset; reduce dose as symptoms improve. Discontinue a few days after resolution of clinical signs (usual total duration: 5 to 7 days); longer duration may be required if therapy is delayed. Migraine, acute treatment: Oral (powder for oral solution or IR tablet): 50 to 100 mg as a single dose . Osteoarthritis: Oral: <i>Diclofenac potassium or sodium formulations</i>: 100 to 150 mg/day in divided doses (may choose immediate release [given in 2 to 4 divided doses], delayed release [given in 2 to 4 divided doses], or extended release [given once daily]); maximum dose: 150 mg/day. Oral: <i>Diclofenac acid</i>: 35 mg 3 times daily; maximum dose: 105 mg/day. Rheumatoid arthritis: Oral: <i>Diclofenac potassium or sodium formulations</i>: 100 mg/day in divided doses (may choose immediate release [given in 2 to 4 divided doses], delayed release [given in 2 to 4 divided doses], or extended release [given once or twice daily]); if needed, may increase to a maximum dose of	dose: 50 mg/dose; maximum daily dose: 3 mg/kg/day; treatment should not exceed 4 days duration.		
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		200 mg/day in divided doses			
Naproxen	Use: Labeled Indications Anti-inflammatory: Relief of the signs and symptoms of gout, ankylosing spondylitis, bursitis, polyarticular juvenile idiopathic arthritis (excluding ER tablets), osteoarthritis, rheumatoid arthritis, and tendinopathy. Delayed-release naproxen is not recommended for initial treatment of acute pain. Dysmenorrhea, primary: Treatment of primary dysmenorrhea. Delayed-release naproxen is not recommended for initial treatment of acute pain. Pain and/or fever: Relief of mild to moderate pain and/or fever. Delayed-release naproxen is not recommended for initial treatment of acute pain. Use: Off-Label: Adult Abnormal uterine bleeding, nonacute; Migraine, acute treatment	Dosing: Adult Anti-inflammatory (eg, for arthritis associated with rheumatic disease): Oral: <i>Immediate release:</i> 250 to 500 mg every 12 hours; maximum dose: 1.5 g/day. <i>Delayed release:</i> 375 to 500 mg every 12 hours; maximum dose: 1.5 g/day. <i>Extended release:</i> 750 mg to 1 g once daily; maximum dose: 1.5 g/day. Note: Some experts generally recommend a maximum dose of 1 g/day for chronic use, except during a disease flare when 1.25 to 1.5 g/day may be considered for several weeks until flare resolves . Dysmenorrhea, primary Oral: <i>Immediate release:</i> Initial: 500 mg once, begin at menses onset or 1 to 2 days prior to onset of menses for severe symptoms; then 500 mg every 12 hours as needed or 250 mg every 6 to 8 hours as needed; maximum dose: 1.25 g/day on day 1, then 1 g/day thereafter; usual duration: 1 to 5 days . <i>Extended release:</i> Initial: 1 g once daily as needed; begin at menses onset or 1 to 2 days prior to onset of menses for severe symptoms; may increase to a maximum of 1.5 g once daily if needed; usual duration: 1 to 5 days . Fever	Dosing: Pediatric Fever: OTC labeling: Children ≥12 years and Adolescents: Immediate release (eg, Aleve): Oral: 200 mg every 8 to 12 hours; if needed may use 400 mg for the initial dose; maximum daily dose: 600 mg/day. Pain, mild to moderate: Children and Adolescents <60 kg: Limited data available: Oral: 5 to 7 mg/kg/dose every 8 to 12 hours; maximum daily dose: 1,000 mg/day Children and Adolescents ≥60 kg: Limited data available: Oral: 5 to 7 mg/kg/dose every 8 to 12 hours; maximum daily dose: 1,000 mg/day . Note: Usual adult dose is 500 mg once, followed by 250 to 500 mg every 12 hours as needed or 250 mg every 6 to 8 hours as needed OTC labeling: Children ≥12 years and Adolescents: Immediate release (eg, Aleve): Oral: 200 mg every 8 to 12 hours; if needed may use 400 mg for the initial dose; maximum daily dose: 600 mg/day. Migraine, treatment: Limited data available: Children >6 years and Adolescents: Immediate release, controlled/delayed release: Oral: 5 to 7 mg/kg/dose every 8 to 12 hours; maximum daily dose: 1,000 mg/day; dosing based on usual analgesic dosing for naproxen and migraine occurrence . Note: In pediatric	Dosing: Kidney Impairment: Adult Altered kidney function: CrCl ≥60 mL/minute: Oral: No dosage adjustment necessary (<1% of unchanged drug excreted in the urine) . CrCl >30 to <60 mL/minute: Oral: No dosage adjustment necessary (<1% of unchanged drug excreted in the urine) . However, use the lowest effective dose for the shortest duration possible. Use of analgesics other than nonsteroidal anti-inflammatory drugs (NSAIDs) or topical NSAIDs may be preferred. Avoid use in patients at high risk for acute kidney injury (ie, volume depleted, hypotensive, elderly, or taking concurrent nephrotoxic medications) CrCl ≤30 mL/minute: Oral: Avoid use due to increased risk of acute kidney injury ; use of analgesics other than NSAIDs or topical NSAIDs are preferred. However, in select patients where alternatives are not effective, after careful assessment of risks versus benefits, use of naproxen may be considered; use the lowest effective dose for the shortest duration possible with close monitoring of kidney function Dosing: Kidney Impairment: Pediatric Children and Adolescents: Oral: eGFR ≥60 mL/minute/1.73 m²:	Contraindications Hypersensitivity to naproxen (eg, anaphylactic reactions, serious skin reactions) or any component of the formulation; history of asthma, urticaria, or allergic-type reactions after taking aspirin or other NSAIDs; use in the setting of coronary artery bypass graft (CABG) surgery Warnings/Precautions Concerns related to adverse effects: CNS effects, Hyperkalemia Disease-related concerns: Aseptic meningitis, Asthma, Hepatic impairment, Renal impairment Warnings: Additional Pediatric Considerations Macrophage activation syndrome has been observed in children and adolescents diagnosed with systemic juvenile idiopathic arthritis on daily nonsteroidal anti-inflammatory drug therapy for extended periods of time

		Oral: <i>OTC labeling (patient-guided therapy): Immediate release:</i> Initial: 200 to 400 mg once, followed by 200 mg every 8 to 12 hours as needed; maximum dose: 400 mg in any 8- to 12-hour period or 600 mg in a 24-hour period. Gout, prophylaxis during initiation of urate-lowering therapy Oral: <i>Immediate release:</i> 250 mg twice daily . <i>Duration of prophylaxis:</i> Patients without tophi: ≥ 3 to 6 months after initiating urate-lowering therapy . Patients with ≥ 1 tophi: Optimal duration is uncertain; some experts continue prophylaxis ≥ 6 months; some patients with severe disease may require >12 months prophylaxis . Gout, treatment, acute flares Oral: <i>Immediate release:</i> Initial: 500 mg twice daily within 24 to 48 hours of flare onset; reduce dose as symptoms improve. Discontinue a few days after resolution of clinical signs (usual total duration: 5 to 7 days); longer duration may be required if therapy is delayed . <i>Extended release:</i> Initial: 1 to 1.5 g once on day 1, followed by 1 g once daily starting on day 2; initiate within 24 to 48 hours of flare onset; reduce dose as symptoms improve. Discontinue a few days after resolution of clinical signs (usual total duration: 5 to 7 days); longer duration may be required if therapy is	patients ≥ 12 years of age, improved efficacy has been shown using a fixed-dose combination with sumatriptan; refer to Sumatriptan and Naproxen monograph for additional information.	No dosage adjustment necessary. eGFR 30 to <60 mL/minute/1.73 m²: There are no specific dosage adjustments provided in the manufacturer's labeling; use with caution and consider using a reduced dose. KDIGO guidelines recommend to temporarily discontinue in patients with intercurrent disease that increases risk of acute kidney injury eGFR <30 mL/minute/1.73 m²: Avoid use.	
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		delayed . Migraine, acute treatment (off-label use): Note: For use as monotherapy in mild to moderate attacks not associated with vomiting or severe nausea; may be used in combination with triptans for severe migraine . Oral: <i>Immediate release</i>: 500 to 750 mg once . Pain Oral:<i>Immediate release</i>: Initial: 500 mg once, followed by 250 to 500 mg every 12 hours as needed or 250 mg every 6 to 8 hours as needed; maximum dose: 1.25 g on day 1, then 1 g/day thereafter. For postoperative pain, doses may be scheduled initially . <i>Extended release</i>: 1 g once daily; may increase to 1.5 g once daily for acute pain, then reduce to a usual maximum dose of 1 g/day. <i>OTC labeling (patient-guided therapy): Immediate release</i>: Initial: 200 to 400 mg once, followed by 200 mg every 8 to 12 hours as needed; maximum dose: 400 mg in any 8- to 12-hour period or 600 mg in a 24-hour period.			
Piroxicam	Use: Labeled Indications Arthritis: Relief of signs and symptoms of osteoarthritis and rheumatoid arthritis.	Dosing: Adult Osteoarthritis, rheumatoid arthritis: Oral: 20 mg once daily. =	Dosing: Pediatric Juvenile idiopathic arthritis (JIA): Limited data available: Children and Adolescents: Fixed dosing : ≤15 kg; Oral: 5 mg once daily.	Dosing: Kidney Impairment: Adult Mild to moderate impairment: There are no dosage adjustments provided in the manufacturer's labeling. Severe impairment: Use is not recommended Dosing: Kidney	Contraindications Hypersensitivity (eg, anaphylactic reactions, serious skin reactions) to piroxicam or to any component of the formulation; patients who have experienced asthma, urticaria, or allergic-type reactions after taking aspirin or other

			16 to <26 kg: Oral: 10 mg once daily. 26 to <46 kg: Oral: 15 mg once daily. ≥46 kg: Oral: 20 mg once daily. Weight-directed dosing: Oral: 0.3 to 0.4 mg/kg/day once daily; maximum daily dose: 20 mg/day	Impairment: Pediatric KDIGO 2012 guidelines provide the following recommendations for NSAIDs (KDIGO 2013): Children and Adolescents: eGFR 30 to <60 mL/minute/1.73 m²: Temporarily discontinue in patients with intercurrent disease that increases risk of acute kidney injury eGFR <30 mL/minute/1.73 m²: Avoid use	nonsteroidal anti-inflammatory drugs; in the setting of coronary artery bypass graft (CABG) surgery Warnings/Precautions Concerns related to adverse effects: • Anaphylactoid reactions • Cardiovascular events • CNS effects • GI events • Hepatic effects • Hyperkalemia • Renal effects • Serum sickness • Skin reactions
Mefenamic acid	Use: Labeled Indications Pain, mild to moderate: Management of mild to moderate pain in adults and pediatric patients ≥14 years of age, when therapy will not exceed 1 week (7 days). Dysmenorrhea, primary: Treatment of primary dysmenorrhea.	Dosing: Adult Pain, mild to moderate: Oral: Initial: 500 mg, then 250 mg every 6 hours as needed; usually not to exceed 1 week. Dysmenorrhea, primary: Oral: Initial: 500 mg beginning at the onset of bleeding and associated symptoms, followed by 250 mg every 6 hours or 500 mg 3 times daily; usually not to exceed 3 days.	Dosing: Pediatric Dysmenorrhea, primary: Postmenarche patients: Oral: Initial: 500 mg beginning at the onset of bleeding and associated symptoms, followed by 250 mg every 6 hours; usual reported duration: 2 to 3 days. Pain, mild to moderate: Adolescents ≥14 years: Oral: Initial: 500 mg once, then 250 mg every 6 hours as needed. Note: Treatment duration generally does not exceed 7 days.	Dosing: Kidney Impairment: Adult Avoid use in patients with preexisting renal disease and in patients with advanced renal disease. KDIGO 2012 guidelines provide the following recommendations for NSAIDs: eGFR 30 to <60 mL/minute/1.73 m²: Temporarily discontinue in patients with intercurrent disease that increases risk of acute kidney injury. eGFR <30 mL/minute/1.73 m²: Avoid use. Dosing: Kidney Impairment: Pediatric Avoid use in patients with preexisting renal disease and in patients with advanced renal disease. KDIGO guidelines	Contraindications Hypersensitivity to mefenamic acid, or any component of the formulation; use in the setting of coronary artery bypass graft (CABG) surgery; history of asthma, urticaria, or allergic-type reactions after taking aspirin or other NSAIDs. Canadian labeling: Additional contraindications (not in US labeling): Pregnancy (third trimester); breast-feeding; severe uncontrolled heart failure; active gastric, duodenal, or peptic ulcer; active gastrointestinal bleeding; cerebrovascular bleeding or other bleeding disorders; inflammatory bowel disease; severe hepatic impairment or active hepatic disease; severe renal impairment (CrCl <30 mL/minute) or deteriorating renal disease; known

				provide the following recommendations for NSAIDs eGFR 30 to <60 mL/minute/1.73 m²: Temporarily discontinue in patients with intercurrent disease that increases risk of acute kidney injury. eGFR <30 mL/minute/1.73 m²: Avoid use.	hyperkalemia; adolescents <18 years of age Warnings/Precautions Concerns related to adverse effects: • Anaphylactoid reactions • Cardiovascular events • CNS effects • Drug reaction with eosinophilia and systemic symptoms • GI events • Hematologic effects • Hepatic effects • Hyperkalemia • Renal effects • Skin reactions
Celecoxib	Use: Labeled Indications Anti-inflammatory: Relief of the signs/symptoms of ankylosing spondylitis or rheumatoid arthritis. Dysmenorrhea: Treatment of primary Juvenile idiopathic arthritis: Relief of the signs/symptoms of juvenile idiopathic arthritis (JIA) in patients 2 years and older. Migraine, acute treatment: Acute treatment of migraine with or without aura in adults Osteoarthritis: Relief of the signs/symptoms of osteoarthritis. Pain, acute: Management of acute pain	Dosing: Adult Anti-inflammatory (eg, for arthritis associated with inflammatory rheumatic disease): Oral: Initial: 200 mg once daily or 100 mg twice daily; may increase to a maximum of 200 mg twice daily for several weeks during a disease flare until the flare resolves. Dysmenorrhea: Oral: Initial: 400 mg, followed by an additional 200 mg approximately 12 hours later, if needed, on day 1; maintenance dose: 200 mg twice daily as needed; maximum daily maintenance dose: 400 mg/day. Begin at menses onset or 1 to 2 days prior to onset of menses for severe symptoms; usual duration: 1 to 5 days Gout, treatment, acute flares (off-label use): Oral: Initial: 400 mg, followed by 200 mg	Dosing: Pediatric Juvenile idiopathic arthritis (JIA): Note: Use the lowest effective dose for the shortest duration of time, consistent with individual patient goals. Children ≥2 years and Adolescents: ≥10 kg to ≤25 kg: Oral: 50 mg twice daily. >25 kg: Oral: 100 mg twice daily.	Dosing: Kidney Impairment: Adult CrCl ≥60 mL/minute: Oral: No dosage adjustment necessary (<1% of the drug excreted in the urine) . CrCl >30 to <60 mL/minute: Oral: No dosage adjustment necessary (<1% of the drug excreted in the urine) . However, use the lowest effective dose for the shortest duration possible. Use of analgesics other than nonsteroidal anti-inflammatory drugs (NSAIDs) or topical NSAIDs may be preferred. Avoid use in patients at high risk for acute kidney injury (ie, volume depleted, hypotensive, elderly, or taking concurrent nephrotoxic medications) . CrCl ≤30 mL/minute: Oral: Avoid use due to increased risk of acute kidney injury ; Use of analgesics other than NSAIDs or topical NSAIDs are	Contraindications Hypersensitivity to celecoxib, sulfonamides, aspirin, other NSAIDs, or any component of the formulation; patients who have experienced asthma, urticaria, or allergic-type reactions after taking aspirin or other NSAIDs; use in the setting of CABG surgery. Canadian labeling: Additional contraindications (not in US labeling): Pregnancy (third trimester); women who are breast-feeding; severe, uncontrolled heart failure; active gastrointestinal ulcer (gastric, duodenal, peptic); active gastrointestinal bleeding; inflammatory bowel disease; cerebrovascular bleeding; severe liver impairment or active hepatic disease; severe renal impairment (CrCl <30

		approximately 12 hours later on day 1, then continue 200 mg twice daily thereafter; maximum daily maintenance dose: 400 mg/day. Initiate within 24 to 48 hours of flare onset; reduce dose as symptoms improve. Discontinue a few days after resolution of clinical signs (usual total duration: 5 to 7 days); longer duration may be required if therapy is delayed Migraine, acute treatment: Note: Limit use to ≤14 days per month to avoid medication-overuse headache .For use as monotherapy in mild to moderate attacks not associated with vomiting or severe nausea Administration early in the course of a migraine attack, at the first sign of pain, may improve response to treatment Oral: 120 mg as a single dose. Maximum: 120 mg per 24 hours Osteoarthritis: Oral: 200 mg/day in 1 or 2 divided doses; use the lowest effective dose for the shortest amount of time. Osteoarthritis: Oral: 200 mg/day in 1 or 2 divided doses; use the lowest effective dose for the shortest amount of time.		preferred. However, in select patients where alternatives are not effective, after careful assessment of risks versus benefits, use of celecoxib may be considered; use the lowest effective dose for the shortest duration possible with close monitoring of kidney function Dosing: Kidney Impairment: Pediatric Children ≥2 years and Adolescents: Oral: Baseline: Mild or moderate impairment: There are no dosage adjustments provided in the manufacturer's labeling; however, since <1% of the drug is excreted in the urine, dosage adjustment is not necessary . Based on unpublished data, AUC was ~40% lower in adult patients with chronic renal insufficiency (GFR 35 to 60 mL/minute) compared with subjects with normal renal function due to a higher apparent clearance. Severe impairment: Use is not recommended. Advanced renal disease: Use is not recommended; however, if celecoxib treatment cannot be avoided, monitor renal function closely.	mL/minute) or deteriorating renal disease; known hyperkalemia; use in patients <18 years of age Warnings/Precautions Concerns related to adverse effects: • Medication-overuse headache • Sulfonamide ("sulfa") allergy Disease-related concerns: • Asthma • Bariatric surgery • Cytochrome P450 isoenzyme 2C9 deficiency • Hepatic impairment • Renal impairment
Etoricoxib	Indications: Ankylosing spondylitis, Rheumatoid arthritis , Osteoarthritis , Acute gouty arthritis , Acute pain , Primary dysmenorrhoea Ref :https://www.mims.com/thailand/drug/info/etoricoxib?mtype=generic	Ankylosing spondylitis, Rheumatoid arthritis Adult: 60 mg once daily, may increase to 90 mg once daily as necessary. Once patient is clinically stable, may reduce dose to 60 mg once daily. Use the lowest effective dose and follow the shortest	Ankylosing spondylitis, Rheumatoid arthritis Child: ≥16 years Same as adult dose. Osteoarthritis Child: ≥16 years Same as adult dose. Acute gouty	Renal Impairment CrCl (mL/min) <30 contraindication	contraindication Hypersensitivity History of allergic-type reactions (e.g. bronchospasm, acute rhinitis, nasal polyps, angioneurotic oedema, or urticaria) after taking aspirin, NSAIDs, or other

		possible duration of treatment. Osteoarthritis Adult: 30 mg once daily, may increase to 60 mg once daily as necessary. Use the lowest effective dose and follow the shortest possible duration of treatment. Acute gouty arthritis Adult: 120 mg once daily. Maximum treatment duration: 8 days. Acute pain Adult: For the treatment of postoperative dental surgery pain: 90 mg once daily. Maximum treatment duration: 3 days. Doses should only be given during the acute symptomatic period. Dosage recommendations may vary among countries or individual products (refer to specific product guidelines). Primary dysmenorrhoea Adult: For the treatment of acute pain: 120 mg once daily. Maximum treatment duration: 8 days. Doses should only be given during the acute symptomatic period. Dosage recommendations may vary among countries or individual products (refer to specific product guidelines).	arthritis Child: ≥16 years Same as adult dose. Acute pain Child: ≥16 years Same as adult dose. Primary dysmenorrhoea Child: ≥16 years Same as adult dose.		cyclooxygenase-2 (COX-2) inhibitors Active peptic ulceration or gastrointestinal haemorrhage Inflammatory bowel disease New York Heart Association (NYHA) class II–IV congestive heart failure Uncontrolled hypertension or persistent high blood pressure (>140/90 mmHg) Established ischaemic heart disease Peripheral arterial disease Cerebrovascular disease (including patients who have recently undergone coronary artery bypass graft surgery or angioplasty) Children and adolescents <16 years Renal impairment (CrCl <30 mL/min) Severe hepatic impairment (Child-Pugh ≥10) Pregnancy and lactation
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Decongestants

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตับ/ไต	ข้อห้ามใช้/ควรระวัง
Phenylephrine + Chlorpheniramine	Use: Labeled Indications Upper respiratory tract conditions:	Dosing: Adult Upper respiratory tract conditions	Dosing: Pediatric Upper respiratory symptoms:	Dosing: Kidney Impairment: Adult Altered kidney	Contraindications OTC labeling: When used for self-medication, do

	Temporary relief of symptoms (nasal congestion; runny nose; sneezing; itching of the eyes, nose, or throat) associated with the common cold, hay fever (allergic rhinitis), and other upper respiratory tract conditions.	Oral: Note: Do not exceed recommended dosage. Chlorpheniramine dosing in terms of chlorpheniramine maleate; phenylephrine dosing in terms of phenylephrine hydrochloride: Liquid: Chlorpheniramine 4 mg and phenylephrine 10 mg per 5 mL: 5 mL every 4 hours (maximum: 30 mL per 24 hours). Tablet: Chlorpheniramine 4 mg and phenylephrine 10 mg: One tablet every 4 hours (maximum: 6 tablets per 24 hours).	<i>Oral Liquid:</i> Chlorpheniramine 4 mg/phenylephrine 10 mg per 5 mL (eg, Ed A-Hist liquid, Nohist LQ): Children 6 years to <12 years: 2.5 mL every 4 hours as needed. Maximum daily dose: 15 mL/24 hours. Children ≥12 years and Adolescents: 5 mL every 4 hours as needed. Maximum daily dose: 30 mL/24 hours. <i>Tablet:</i> Chlorpheniramine 4 mg/phenylephrine 10 mg per tablet: Children 6 years to <12 years: $\frac{1}{2}$ tablet every 4 hours as needed. Maximum daily dose: 3 tablets/24 hours. Children ≥12 years and Adolescents: 1 tablet every 4 to 6 hours as needed. Maximum daily dose: 6 tablets/24 hours.	function: eGFR >30 mL/minute/1.73 m²: No dosage adjustment necessary eGFR ≤30 mL/minute/1.73 m²: No initial dosage adjustment necessary; however, greatly prolonged half-lives of chlorpheniramine in patients with severe kidney disease have been reported; use with caution Dosing: Kidney Impairment: Pediatric There are no dosage adjustments provided in the manufacturer's labeling.	not use with or within 2 weeks of discontinuing monoamine oxidase inhibitor; to sedate a child. Warnings/Precautions <i>Concerns related to adverse effects:</i> • CNS depression: May cause CNS depression, which may impair physical or mental abilities; patients must be cautioned about performing tasks which require mental alertness <i>Special populations:</i> • Older adult: Use with caution in older adults; may be more sensitive to adverse effects. • Pediatric: Antihistamines may cause excitation in young children.
Phenylephrine + Brompheniramine	Use: Labeled Indications Upper respiratory tract conditions: Temporary relief of symptoms (nasal congestion, runny nose, sneezing, itchy nose or throat, itchy/watery eyes, nasal passages swelling) associated with the common cold, allergic rhinitis (hay fever), and other upper respiratory tract conditions.	Dosing: Adult Upper respiratory tract conditions Oral: Note: These are general dosage guidelines; also refer to product specific dosing. All dosing is presented in terms of brompheniramine maleate and phenylephrine hydrochloride: Liquid: Brompheniramine 1 mg and phenylephrine 2.5 mg per 5 mL: 20 mL every 4 hours (maximum: 120 mL per 24 hours). Brompheniramine 2 mg and phenylephrine 5 mg per 5 mL: 10 mL	Dosing: Pediatric Cough and upper respiratory allergy symptoms: Liquid: Brompheniramine 1 mg/phenylephrine 2.5 mg per 5 mL (eg, Dimetapp, Dimaphen, Rynex PE): Oral: Children 6 to <12 years: 10 mL every 4 hours as needed; maximum daily dose: 60 mL/24 hours. Children ≥12 years and Adolescents: 20 mL every 4 hours as needed; maximum daily dose: 120 mL/24 hours. Brompheniramine 2 mg/phenylephrine 5 mg per 5 mL (eg,	Dosing: Kidney Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling. However, need for dosage adjustment unlikely as high molecular weight precludes filtration at the glomerulus Dosing: Kidney Impairment: Pediatric There are no dosage adjustments provided in the manufacturer's labeling.	Contraindications OTC labeling: When used for self-medication, do not use to sedate a child, or with or within 14 days of stopping monoamine oxidase inhibitor therapy. Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information. Warnings/Precautions <i>Concerns related to adverse effects:</i> • CNS depression: May cause CNS depression, which may impair physical

		every 4 hours (maximum: 60 mL per 24 hours). Brompheniramine 4 mg and phenylephrine 10 mg per 5 mL: 5 mL every 4 hours (maximum: 30 mL per 24 hours). Tablets: Brompheniramine 4 mg and phenylephrine 10 mg per tablet: One tablet every 4 hours (maximum: 6 tablets per 24 hours).	Maxi-Tuss PE): Oral: Children 6 to <12 years: 5 mL every 4 hours as needed; maximum daily dose: 30 mL/24 hours. Children ≥12 years and Adolescents: 10 mL every 4 hours as needed; maximum daily dose: 60 mL/24 hours. Brompheniramine 4 mg and phenylephrine 10 mg per 5 mL (eg, Glenmax PEB): Oral: Children ≥12 years and Adolescents: 5 mL every 4 hours; maximum daily dose: 30 mL/24 hours. Tablets: Brompheniramine 4 mg and phenylephrine 10 mg per tablet (eg, Ru-Hist D): Oral: Children 6 to <12 years: 1/2 tablet every 4 hours as needed; maximum daily dose: 3 tablets/24 hours (6 doses). Children ≥12 years and Adolescents: 1 tablet every 4 hours as needed; maximum daily dose: 6 tablets/24 hours.		or mental abilities; patients must be cautioned about performing tasks that require mental alertness (eg, operating machinery, driving). <i>Special populations:</i> • Pediatric: Antihistamines may cause excitation in young children.
Oxymetazoline	Use: Labeled Indications Nasal congestion: Temporary relief of nasal congestion (due to a cold, hay fever, or other upper respiratory allergies) and sinus congestion/pressure.	Dosing: Adult Nasal congestion Nasal congestion: Intranasal: Instill 2 to 3 sprays into each nostril twice daily (maximum dose: 2 doses/24 hours). <i>Duration:</i> Generally limit to 3 to 5 days of consecutive use due to risk of rhinitis medicamentosa . In patients with persistent symptoms despite use of first-line therapies, some guidelines suggest oxymetazoline may be offered in combination with an	Dosing: Pediatric Nasal congestion: Note: Therapy should not exceed 3 days. Children 2 to <6 years: 0.025% solution: Intranasal: Instill 2 to 3 sprays into each nostril twice daily as needed; doses should be separated by 10 to 12 hours; maximum daily dose: 2 doses/24 hours. Children ≥6 years and Adolescents: 0.05% solution: Intranasal: Instill 2 to 3 sprays into each nostril twice daily; doses should be	Dosing: Kidney Impairment: Adult There are no dosage adjustments provided in manufacturer's labeling. Dosing: Kidney Impairment: Pediatric There are no dosage adjustments provided in manufacturer's labeling.	Contraindications OTC labeling: When used for self-medication, do not use for more than 3 days. Warnings/Precautions <i>Concerns related to adverse effects:</i> • Local nasal effects: Temporary discomfort such as burning, stinging, sneezing, or an increased nasal discharge may occur. • Rebound nasal congestion: Frequent or prolonged use

		intranasal corticosteroid for up to 4 weeks with low risk of rhinitis medicamentosa	separated by 10 to 12 hours; maximum daily dose: 2 doses/24 hours.		may cause nasal congestion to recur or worsen. Disease-related concerns: • Cardiovascular disease: Use with caution in patients with hypertension or heart disease. • Diabetes: Use with caution in patients with diabetes mellitus. • Thyroid disease: Use with caution in patients with thyroid disease. • Prostatic hyperplasia/urinary obstruction: Use with caution in patients with prostatic hyperplasia and/or GU obstruction.
Xylometazoline	Use: Labeled Indications Nasal Congestion: Temporary relief of nasal and nasopharyngeal mucosal congestion	Dosing: Adult Nasal congestion: Intranasal: Spray: 2 sprays in each nostril every 8 to 10 hours (maximum: 6 sprays in each nostril daily).	✗	Dosing: Kidney Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling.	Contraindications Hypersensitivity to xylometazoline or any component of the formulation; patients with transphenoidal hypophysectomy or surgery exposing the dura mater; narrow angle glaucoma; rhinitis sicca or atrophic rhinitis Warnings/Precautions Concerns related to adverse effects: • Rebound nasal congestion: Frequent or prolonged use may cause nasal congestion to recur or worsen. Disease-related concerns: • Cardiovascular disease: Use with caution in patients with hypertension or heart disease. • Diabetes mellitus: Use with caution in patients with diabetes mellitus. • Thyroid disease: Use with caution in patients with thyroid

					disease. • Prostatic hyperplasia/Urinary obstruction: Use with caution in patients with prostatic hyperplasia and/or GU obstruction.
Naphazoline HCl	Use: Labeled Indications Decrease in eye redness (vasoconstrictor): Relief of redness of the eye due to minor irritation; temporary relief of burning and irritation due to dry eyes; as a protectant against further irritation or dryness of the eye.	Dosing: Adult Decrease in eye redness (vasoconstrictor): Ophthalmic: 1 to 2 drops into affected eye(s) up to 4 times daily.	Dosing: Pediatric Ocular redness, irritation: Limited data available: Children ≥6 years and Adolescents: Ophthalmic solution 0.012% to 0.03% (OTC products): Instill 1 to 2 drops in affected eye up to four times daily	Dosing: Kidney Impairment: Adult There are no dosage adjustments provided in the manufacturer's labeling and limited pharmacokinetic data are available. Dosing: Kidney Impairment: Pediatric There are no dosage adjustments provided in the manufacturer's labeling.	Warnings/Precautions Disease-related concerns: • Cardiovascular disease: Use with caution in patients with cardiovascular abnormalities or hypertension. • Diabetes: Use with caution in patients with diabetes mellitus. • Hyperthyroidism: Use with caution in patients with hyperthyroidism. • Infection/injury: Use with caution in patients with ophthalmic infection or injury.

Intranasal steroid

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตับ/ไต	ข้อห้ามใช้/ควรระวัง
Mometasone furoate	• Allergic rhinitis, seasonal: Relief of hay fever and other upper respiratory allergies (eg, nasal congestion, runny nose, sneezing, itchy nose) in adults and children ≥2 years of age • Off-label use: Adult Nonallergic rhinitis; Rhinosinusitis, acute bacterial; Rhinosinusitis, chronic, without nasal polyps	Clinical considerations: For patients with mucous crusting, rinsing with saline nasal spray 10 to 15 minutes prior to mometasone administration can remove mucous crusting and improve nasal coating (Ref). If nasal obstruction is so severe that sprays cannot penetrate when treating rhinitis, consider concomitant use of an intranasal decongestant for up to 5 days Rhinosinusitis, acute bacterial: 2 sprays in each nostril twice daily (total daily dose: 400	×	No dosage adjustment	• Contraindications ; Hypersensitivity to mometasone or any component of the formulation. When used for self-medication, do not use to treat asthma, if nose injury or surgery is not fully healed, or in children <2 years of age. • Warnings/Precautions • Adrenal suppression: When recommended doses are exceeded, or in extremely sensitive individuals, may cause hypercortisolism or suppression of hypothalamic-pituitary

		mcg); if inadequate symptom control, may increase to 4 sprays in each nostril twice daily (total daily dose: 800 mcg) Rhinosinusitis, chronic, with nasal polyps: Note: A 1- to 3-month course of nasal corticosteroids is part of a multimodal approach (including intranasal saline) to initial treatment of chronic rhinosinusitis; also used as part of multimodal for maintenance therapy after symptom control is achieved Intranasal (50 mcg/spray): 2 sprays in each nostril twice daily; 2 sprays (100 mcg) in each nostril once daily may be effective in some patients. Maximum daily dose: 400 mcg. Rhinosinusitis, chronic, without nasal polyps (off-label use): Intranasal (50 mcg/spray): 2 sprays in each nostril once or twice daily (maximum daily dose: 400 mcg)			y-adrenal (HPA) axis. Reports consistent with hypercortisolism are rare. HPA axis suppression may lead to adrenal crisis. Withdrawal and discontinuation of a corticosteroid should be done slowly and carefully. Fatalities have occurred due to adrenal insufficiency in asthmatic patients during and after transfer from systemic corticosteroids to aerosol steroids; aerosol steroids do not provide the systemic steroid needed to treat patients having trauma, surgery, or infections. • Delayed wound healing: Avoid nasal corticosteroid use in patients with recent nasal septal ulcers, nasal surgery, or nasal trauma until healing has occurred. • Hypersensitivity reactions: Hypersensitivity reactions, including rash, pruritus, angioedema, and wheezing, have been reported; discontinue if such reactions occur. • Immunosuppression: Prolonged use of corticosteroids may also increase the incidence of secondary infection, mask acute infection (including fungal infections), prolong or exacerbate viral infections, or limit response to vaccines. Avoid exposure to chickenpox and/or measles, especially if not immunized. • Local nasal effects: Nasal septum perforation, epistaxis, irritation, and infection of the nose and/or pharynx may occur. Periodically examine nasal mucosa; discontinuation of therapy may be necessary if an
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					infection occurs. Disease-related concerns: • Hepatic impairment: Use with caution in patients with hepatic impairment: drug accumulation may increase with severity of hepatic impairment. • Infections: Use with caution or avoid use in patients with tuberculosis (TB) disease (active TB) or infection (latent TB) of the respiratory tract; untreated systemic fungal, bacterial, viral, or parasitic infections; or ocular herpes simplex. Do not use in untreated localized infection involving the nasal mucosa; concurrent antimicrobial therapy should be administered if bacterial infection of the sinuses is suspected/confirmed. • Ocular disease: Use with caution in patients with cataracts and/or glaucoma; increased intraocular pressure, glaucoma, and cataracts have occurred with prolonged use. Consider routine eye exams in chronic users or in patients who report visual changes.
Fluticasone furoate	Use: Labeled Indications • Allergic rhinitis: Management of seasonal and perennial allergic rhinitis in adults and pediatric patients ≥4 years of age (50 mcg/spray) or ≥2 years of age (27.5 mcg/spray). • Nonallergic rhinitis: Management of the nasal symptoms of perennial nonallergic rhinitis in adults and	Clinical considerations: For patients with mucous crusting, rinsing with saline nasal spray 10 to 15 minutes prior to mometasone administration can remove mucous crusting and improve nasal coating (Ref). If nasal obstruction is so severe that sprays cannot penetrate when treating rhinitis, consider concomitant use of an intranasal decongestant for up to 5 days	✗	No dosage adjustment	• Contraindications ; Hypersensitivity to fluticasone or any component of the formulation. When used for self-medication, do not use in children <4 years of age (Flonase Allergy Relief) or children <2 years of age (Flonase Sensimist), for the treatment of asthma, or with current injury or surgery to nose that is not fully healed. • Warnings/

	pediatric patients ≥ 4 years of age (50 mcg/spray). Rhinosinusitis, chronic, with nasal polyps: Treatment of chronic rhinosinusitis with nasal polyps in adults (93 mcg/spray). Rhinosinusitis, chronic, without nasal polyps: Treatment of chronic rhinosinusitis without nasal polyps in adults (93 mcg/spray).	Rhinosinusitis, chronic, with nasal polyps: Note: A 1- to 3-month course of nasal corticosteroids is part of a multimodal approach (including intranasal saline) to initial treatment of chronic rhinosinusitis. Also used as part of multimodal maintenance therapy after symptom control is achieved. Fluticasone furoate (27.5 mcg/spray) (off-label use): Intranasal: Initial: 2 sprays in each nostril once daily. Some experts recommend increasing to 2 sprays in each nostril twice daily, if needed. Rhinosinusitis, chronic, without nasal polyps: Fluticasone furoate (27.5 mcg/spray) (off-label use): Intranasal: Initial: 2 sprays in each nostril once daily (maximum dose: 2 sprays per nostril per day)		Precautions • Adrenal suppression: May cause hypercortisolism or suppression of hypothalamic-pituitary-adrenal (HPA) axis, particularly in younger children or in patients receiving high doses for prolonged periods. HPA axis suppression may lead to adrenal crisis. Withdrawal and discontinuation of a corticosteroid should be done slowly and carefully. Pediatric patients may be more susceptible to systemic toxicity. Particular care is required when patients are transferred from systemic corticosteroids to inhaled products due to possible adrenal insufficiency or withdrawal from steroids, including an increase in allergic symptoms. Adult patients receiving ≥ 20 mg per day of prednisone (or equivalent) may be most susceptible. Concurrent use of ritonavir (and potentially other strong inhibitors of CYP3A4) may increase fluticasone levels and effects on HPA suppression. • Delayed wound healing: Avoid use in patients with recent nasal septal ulcers, nasal surgery, or nasal trauma until healing has occurred. • Hypersensitivity: Hypersensitivity reactions (including anaphylaxis, angioedema, rash, contact dermatitis, hypotension, bronchospasm, and urticaria) have been reported; discontinue for severe reactions. • Immunosuppression : Prolonged use of corticosteroids may also increase the incidence of
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					secondary infection, mask acute infection (including fungal infections), prolong or exacerbate viral infections, or limit response to vaccines. Exposure to chickenpox and/or measles should be avoided, especially if not immunized; if the patient is exposed, prophylaxis with varicella zoster immune globulin or pooled intramuscular immunoglobulin, respectively, may be indicated; if chickenpox develops, treatment with antiviral agents may be considered. • Local nasal effects: Nasal septal perforation, nasal ulceration, nasal erosion, epistaxis, and localized Candida albicans infections of the nose and/or pharynx may occur. Monitor patients periodically for adverse nasal effects; discontinuation of therapy may be necessary if an infection occurs; discontinue therapy if nasal septal perforation occurs Disease-related concerns: • Hepatic impairment: Use with caution in patients with moderate to severe hepatic impairment (accumulation may occur); monitor patients closely. • Infections: Use caution or avoid use in patients with tuberculosis (TB) infection (latent TB) or disease (active TB) of the respiratory tract, systemic fungal, bacterial, viral, or parasitic infections, or ocular herpes simplex. Do not use in untreated localized infection involving the nasal mucosa; concurrent antimicrobial therapy should be administered if bacterial infection of
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					the sinuses is suspected/confirmed. • Ocular disease: Use with caution in patients with cataracts and/or glaucoma; increased intraocular pressure, open-angle glaucoma, and cataracts have occurred with prolonged use. Consider routine eye exams in long-term users or in patients who report visual changes.
Fluticasone propionate	Use: Labeled Indications • Allergic rhinitis: Management of seasonal and perennial allergic rhinitis in adults and pediatric patients ≥4 years of age (50 mcg/spray) or ≥2 years of age (27.5 mcg/spray). Nonallergic rhinitis: Management of the nasal symptoms of perennial nonallergic rhinitis in adults and pediatric patients ≥4 years of age (50 mcg/spray). Rhinosinusitis, chronic, with nasal polyps: Treatment of chronic rhinosinusitis with nasal polyps in adults (93 mcg/spray). Rhinosinusitis, chronic, without nasal polyps: Treatment of chronic rhinosinusitis without nasal polyps in adults (93 mcg/spray).	Clinical considerations: For patients with mucous crusting, rinsing with saline nasal spray 10 to 15 minutes prior to mometasone administration can remove mucous crusting and improve nasal coating (Ref). If nasal obstruction is so severe that sprays cannot penetrate when treating rhinitis, consider concomitant use of an intranasal decongestant for up to 5 days Rhinosinusitis, chronic, with nasal polyps: Note: A 1- to 3-month course of nasal corticosteroids is part of a multimodal approach (including intranasal saline) to initial treatment of chronic rhinosinusitis. Also used as part of multimodal maintenance therapy after symptom control is achieved. Fluticasone propionate (50 mcg/spray) (off-label use): Intranasal: Initial: 2 sprays in each nostril once daily or 1 spray in each nostril twice daily. Doses up to 2 sprays in each nostril twice daily have been	✗	No dosage adjustment	เหมือนกับ Fluticasone furoate

		studied Rhinosinusitis, chronic, without nasal polyps: Fluticasone propionate (50 mcg/spray) (off-label use): Intranasal: Initial: 2 sprays in each nostril once daily or 1 spray in each nostril twice daily (maximum dose: 2 sprays per nostril per day)			
Budesonide	Use: Off-Label: Adult Nonallergic rhinitis; Rhinosinusitis, chronic	• Rhinosinusitis, chronic, with nasal polyps: 32 mcg/spray formulation (off-label use): Intranasal: 2 sprays (64 mcg) in each nostril once or twice daily (total daily dose: 128 to 256 mcg/day) or 4 sprays (128 mcg) in each nostril once daily (total daily dose: 256 mcg/day) Rhinosinusitis, chronic, without nasal polyps (off-label use): Intranasal: 2 sprays (64 mcg) in each nostril twice daily (total daily dose: 256 mcg/day)	×	No dosage adjustment	• Contraindications US labeling: When used for self-medication (OTC use), do not use if known hypersensitivity to budesonide or any component of the formulation or in children <6 years of age. • Warnings/Precautions • Adrenal suppression: May cause hypercortisolism or suppression of hypothalamic-pituitary-adrenal (HPA) axis, particularly in younger children or in patients receiving high doses for prolonged periods. HPA axis suppression may lead to adrenal crisis. Withdrawal and discontinuation of a corticosteroid should be done slowly and carefully. Pediatric patients may be more susceptible to systemic toxicity. Particular care is required when patients are transferred from systemic corticosteroids to inhaled products due to possible adrenal insufficiency or withdrawal from steroids, including an increase in allergic symptoms. Adult patients receiving ≥20 mg per day of prednisone (or equivalent) may be most susceptible.

					• Delayed wound healing: Avoid nasal corticosteroid use in patients with recent nasal septal ulcers, nasal surgery or nasal trauma until healing has occurred. • Hypersensitivity: Hypersensitivity reactions (eg, anaphylactic reactions, angioedema, pruritus, urticaria, rash, dermatitis) may occur. • Immunosuppression: Prolonged use of corticosteroids may also increase the incidence of secondary infection, mask acute infection (including fungal infections), prolong or exacerbate viral infections, or limit response to vaccines. Avoid exposure to chickenpox and/or measles, especially if not immunized. Avoid use or use with caution in patients with tuberculosis (TB) infection (latent TB) or disease (active TB), untreated bacterial or fungal infections (local or systemic), viral or parasitic infections, or ocular herpes simplex. The Canadian labeling contraindicates use in patients with TB infection or disease, or with untreated bacterial, fungal, or viral infections. • Local nasal effects: Nasal septal perforation, nasal ulceration, epistaxis, and localized Candida albicans infections of the nose and/or pharynx may occur. Monitor patients periodically for adverse nasal effects; discontinuation of therapy may be necessary if an infection occurs. Disease-related
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					concerns: • Ocular disease: Use with caution in patients with cataracts and/or glaucoma; increased intraocular pressure, glaucoma, and cataracts have occurred with prolonged use. Consider routine eye exams in chronic users or in patients who report visual changes.
Triamcinolone acetonide	Use: Off-Label: Adult Acute bacterial rhinosinusitis, adjunct to antibiotics (empiric treatment); Chronic rhinosinusitis; Nonallergic rhinitis	• Acute bacterial rhinosinusitis, adjunctive therapy (empiric treatment) (off-label use): Note: Dosing recommendations based on general dosing range in manufacturer's labeling. Intranasal: Two sprays (110 mcg) in each nostril once daily. • Rhinosinusitis, chronic (off-label use): Note: Dosing recommendations based on general dosing range in manufacturer's labeling. Intranasal: Two sprays (110 mcg) in each nostril once daily.	✗	No dosage adjustment	Contraindications Hypersensitivity to triamcinolone or any component of the formulation OTC labeling: When used for self-medication, do not use in children <2 years of age. Warnings/Precautions • Adrenal suppression: When used at excessive doses, may cause hypercortisolism or suppression of hypothalamic-pituitary-adrenal (HPA) axis, particularly in younger children or in patients receiving high doses for prolonged periods. HPA axis suppression may lead to adrenal crisis. Withdrawal and discontinuation of a corticosteroid should be done slowly and carefully. Particular care is required when patients are transferred from systemic corticosteroids to inhaled products due to possible adrenal insufficiency or withdrawal from steroids, including an increase in allergic symptoms. • Delayed wound healing: Avoid use in patients with recent nasal septal ulcers, nasal surgery, or nasal trauma until

					healing has occurred. • Immunosuppression: Prolonged use of corticosteroids may increase the incidence of secondary infection, mask acute infection (including fungal infections), prolong or exacerbate viral infections, or limit response to vaccines. Exposure to chickenpox and/or measles should be avoided. • Local nasal effects: Nasal septal perforation, nasal ulceration, epistaxis, and localized Candida albicans infections of the nose and/or pharynx may occur. Monitor patients periodically for adverse nasal effects. Disease-related concerns: • Infections: Use caution or avoid in patients with ocular herpes simplex, tuberculosis (TB) infection (latent TB), and/or TB reactivity, or in patients with untreated fungal, viral, or bacterial infections. • Ocular disease: Use with caution in patients with cataracts and/or glaucoma; increased intraocular pressure, open-angle glaucoma, and cataracts have occurred with prolonged use. Consider routine eye exams in chronic users.
Beclometasone dipropionate	Use: Off-Label: Adult Acute bacterial rhinosinusitis, adjunct to antibiotics (empiric treatment); Chronic Rhinosinusitis				
Ciclesonide	Use: Off-Label:				

	Adult Acute bacterial rhinosinusitis, adjunct to antibiotics (empiric treatment); Chronic Rhinosinusitis				
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Antitussive/ Mucolytics/ Expectorants

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตับ/ไต	ข้อห้ามใช้/ควรระวัง
Dextromethorphan	• Use: Labeled Indications Cough (suppressant): Temporary control of cough due to minor throat and bronchial irritation associated with the common cold or inhaled irritants; temporary relief of cough impulse to improve sleep (extended release formulations)	Oral: 10 to 20 mg every 4 hours or 20 to 30 mg every 6 to 8 hours; extended release: 60 mg twice daily; maximum: 120 mg/24 hours	Oral syrup (immediate release): Children 2 to <6 years: 5 mg every 4 hours as needed; do not exceed 6 doses in 24 hours. Children 6 to <12 years: 10 mg every 4 hours as needed; do not exceed 6 doses in 24 hours. Children ≥12 years and Adolescents: 20 mg every 4 hours as needed; do not exceed 6 doses in 24 hours. Long-acting liquid: Children ≥12 years and Adolescents: 30 mg every 6 to 8 hours as needed; do not exceed 4 doses in 24 hours. Oral capsule: Children ≥12 years and Adolescents: 30 mg every 6 to 8 hours as needed; do not exceed 4 doses in 24 hours. Extended-release suspension (dextromethorphan polistirex): Children 4 to <6 years: 15 mg every 12 hours as needed; do not exceed 30 mg in 24 hours. Children 6 to <12 years: 30 mg every 12 hours as needed; do not exceed 60 mg in 24 hours. Children ≥12 years and Adolescents: 60 mg every 12 hours as needed; do not exceed 120 mg in 24	No dosage adjustment	Contraindications Concurrent administration with or within 2 weeks of discontinuing an MAO inhibitor Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information. Warnings/ Precautions • Serotonin syndrome: Symptoms of agitation, confusion, hallucinations, hyper-reflexia, myoclonus, shivering, and tachycardia may occur with concomitant proserotonergic drugs (ie, SSRIs/SNRIs or triptans); especially with higher dextromethorphan doses. Special populations: • CYP2D6 poor metabolizers: Dextromethorphan is metabolized by hepatic CYP2D6. Poor metabolizers of CYP2D6 may have exaggerated or prolonged effects of dextromethorphan. Increased risk may be seen with concomitant use of potent CYP2D6 inhibitors; use with caution (Abduljalil 2010; Jurica 2012; Sager 2014; Zhou 2009).

			hours.		• Debilitated patients: Use with caution in patients who are sedated, debilitated or confined to a supine position. • Pediatric: Use with caution in atopic children. Not for OTC use in children <4 years of age. Dosage form specific issues: • Benzyl alcohol and derivatives: Some dosage forms may contain sodium benzoate/benzoic acid; benzoic acid (benzoate) is a metabolite of benzyl alcohol; large amounts of benzyl alcohol (≥ 99 mg/kg/day) have been associated with a potentially fatal toxicity ("gasping syndrome") in neonates; the "gasping syndrome" consists of metabolic acidosis, respiratory distress, gasping respirations, CNS dysfunction (including convulsions, intracranial hemorrhage), hypotension, and cardiovascular collapse (AAP ["Inactive" 1997]; CDC, 1982); some data suggests that benzoate displaces bilirubin from protein binding sites (Ahlfors, 2001); avoid or use dosage forms containing benzyl alcohol derivative with caution in neonates. See manufacturer's labeling. • Tartrazine: Some products may contain tartrazine. • Abuse/misuse: Healthcare providers should be alert to problems of abuse or misuse. Abuse can cause death, brain damage, seizure, loss of consciousness, and irregular heartbeat. • Self-medication
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					(OTC use): When used for self medication (OTC) notify healthcare provider if symptoms do not improve within 7 days, or are accompanied by fever, rash or persistent headache. Do not use for persistent or chronic cough (as with smoking, asthma, chronic bronchitis, emphysema) or if cough is accompanied by excessive phlegm unless directed to do so by healthcare provider.
Levodropropizine	Cough suppressant (สำหรับอาการไอแห้ง) Ref: MIMs Thailand	- For symptomatic relief: As conventional tab/ Oral syrup: 60 mg up to three times/day given at intervals of at least 6 hours. - As controlled-release tab: 90 mg two times/day	Children 2-12 years: as oral syrup: 1 mg/kg three times/day given at intervals of at least 6 hours Children > 12 years: as conventional tab/oral syrup: same as adult dose	Hepatic severe impairment : Contraindicated	- Significant: Rarely, drowsiness or altered consciousness. - Cardiac disorders: Palpitations. - Gastrointestinal disorders: Nausea, vomiting, diarrhoea, abdominal pain, heartburn. - General disorders and administration site conditions: Asthenia, fatigue. - Nervous system disorders: Somnolence, dizziness, headache.
Carbocysteine	Adjunctive mucolytic therapy in respiratory tract disorders characterized by excessive or viscous mucous secretions	Oral Adolescents ≥15 years and Adults: 750 mg to 2.25 g/day in 3 to 4 divided doses.	Oral Children 2 to 5 years: 100 mg twice daily or 62.5 to 125 mg 4 times daily. Children 6 to 11 years: 100 to 250 mg 3 times daily. Children 12 to <15 years: 100 to 750 mg 3 times daily.	×	Contraindications : Active peptic or duodenal ulcer. Children <2 years. Special Precautions : Patient with history of gastroduodenal ulceration, chronic bronchial asthma, respiratory failure. Patients taking concomitant medicines known to cause gastrointestinal bleeding. It is not recommended to be concomitantly used with antitussives and/or medicines that dry up bronchial secretions. Children (≥2 years) and

					elderly. Pregnancy and lactation. Adverse Reactions Significant: Gastrointestinal bleeding. Gastrointestinal disorders: Nausea, vomiting, diarrhoea, epigastric discomfort. Immune system disorders: Anaphylactic reactions, fixed drug eruption. Skin and subcutaneous tissue disorders: Rash. Rarely, bullous dermatitis (e.g. Stevens-Johnson syndrome, erythema multiforme).
Acetylcysteine (N-Acetylcysteine)	Use: Labeled Indications Acetaminophen overdose: To prevent or lessen hepatic injury after ingestion of a potentially hepatotoxic quantity of acetaminophen in patients with acute ingestion or from repeated supratherapeutic ingestion (RSTI). Mucolytic: Adjunct therapy in patients with abnormal, viscid, or inspissated mucous secretions in conditions such as: chronic bronchopulmonary disease (chronic emphysema, emphysema with bronchitis, chronic asthmatic bronchitis, tuberculosis, bronchiectasis, primary amyloidosis of the lung); acute bronchopulmonary disease (pneumonia, bronchitis, tracheobronchitis); pulmonary complications of cystic fibrosis; tracheostomy care; pulmonary complications associated with surgery; use during anesthesia; posttraumatic chest conditions; atelectasis due to	Oral Mucolytic Adult: As adjunctive therapy for respiratory tract disorders associated with excessive, viscous mucus: As granules for oral solution: 200 mg 2-3 times daily. As effervescent tab: 600 mg once daily. Treatment duration may vary depending on the nature and severity of the disease. Dosage recommendations may vary among countries and individual products (refer to specific product guidelines). Ref: https://www.mims.com/thailand/drug/info/a/cetylcysteine?mttype=g	Oral Mucolytic Child: 2-6 years As granules for oral solution: 100 mg 2-4 times daily; >6 years Same as adult dose. Treatment or dosage recommendations may vary among countries and individual products (refer to specific product guidelines). Ref: https://www.mims.com/thailand/drug/info/a/cetylcysteine?mttype=g	✗	• Contraindications ; For mucolytic use (oral): Children <2 years. Special Precautions; Patient with asthma, history of bronchospasm, atopy; decreased cough reflex (e.g. frail patient), predisposition to gastrointestinal haemorrhage [e.g. oesophageal varices, peptic ulcer] (oral). Patient requiring fluid restriction or who weighs ≤40 kg (IV). Acetylcysteine is available in multiple dosage forms, refer to detailed product guidelines for specific recommendations. Children. Pregnancy and lactation. Adverse Reactions Significant: Increased risk of upper gastrointestinal haemorrhage (oral). Rarely, bronchospasm; serious skin reactions (e.g. Stevens-Johnson syndrome, Lyell's syndrome) with mucolytic use (oral).

	mucous obstruction; diagnostic bronchial studies (bronchograms, bronchospirometry, bronchial wedge catheterization).				
Bromhexine	Acute and chronic bronchopulmonary diseases associated with abnormal mucous secretion and impaired mucous transport	Adults: 8 to 16 mg 3 times daily (maximum: 48 mg/day).	Use of bromhexine in children <6 years may be contraindicated in some countries. Children 2 to <6 years: 2 mg 3 times daily or 4 mg twice daily (maximum: 8 mg/day). Children 6 to <12 years: 4 to 8 mg 3 times daily (maximum: 24 mg/day). Children ≥12 years, Adolescents : 8 to 16 mg 3 times daily	✗	Special Precautions ; Patient with existing or history of gastric ulceration; asthma. Severe renal and hepatic impairment. Children. Pregnancy and lactation. Adverse Reactions Significant: Rarely, Stevens-Johnson syndrome, toxic epidermal necrolysis (TEN), acute generalised exanthematous pustulosis. - Gastrointestinal disorders: Diarrhoea, nausea, vomiting, upper abdominal pain. - Immune system disorders: Hypersensitivity reaction (including anaphylactic reaction and shock, angioedema, bronchospasm). - Investigations: Increased serum aminotransferase (AST), ALT. - Nervous system disorders: Headache, dizziness. - Skin and subcutaneous tissue disorders: Rash, urticaria, pruritus.
Ambroxol	Treatment of respiratory tract disorders associated with viscid mucus; relief of sore throat symptoms (lozenge only)	Adults: 60 to 120 mg/day in 2 to 3 divided doses (immediate release) or 75 mg once daily (extended release capsules; adults only).	Children 2 to <6 years: 7.5 to 15 mg 3 times daily. Children 6 to 12 years: 15 to 30 mg 2 to 3 times daily. Children >12 years, Adolescents: 60 to 120 mg/day in 2 to 3 divided doses	✗	Special Precautions ; Patient with existing or history of gastric ulceration; asthma. Severe renal and hepatic impairment. Children. Pregnancy and lactation. Adverse Reactions

			(immediate release) or 75 mg once daily (extended release capsules; adults only).		Significant: Hypersensitivity reactions (e.g. rash, pruritus, urticaria, anaphylactic shock, angioedema). Gastrointestinal disorders: Diarrhoea, dyspepsia, nausea, vomiting, abdominal pain, heartburn, bloated feeling, dry mouth or throat and dysgeusia, oral and pharyngeal hypoesthesia. Potentially Fatal: Rarely, Stevens-Johnson syndrome and toxic epidermal necrolysis.
Terco-D (Dextromethorphan 15 mg + Glyceryl guaiacolate 100 mg + Terpin hydrate 130 mg)	• As expectorant & relief of cough due to common cold, bronchitis. Ref: MIMs Thailand	Adult : 1 tab 3-4 times/day	Children > 12 years : 1 tab 3-4 times/day	✗	• Adverse reactions: Mild drowsiness, headache, rash, nausea, vomiting, confusion, constipation, GI disturbance, stomach pain, mild dizziness, urticaria. • Special Precautions: Not to be used for persistent or chronic cough eg, w/ smoking, emphysema, asthma or cough accompanied by excessive secretions; in patients who received MAOI or 2 wk after discontinuing MAOI. Not to be given during pregnancy. Lactation.

Throat sprays/ lozenges/ syrup/Gargle

ชื่อยา/ชื่อการค้า	ชื่อยา	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตัว/โต	ข้อห้ามใช้/ควรระวัง
Kamilosan M spray	Kamillosan®M	ขนาดและวิธีใช้	วิธีใช้	✗	ข้อควรระวัง

	คามิลโลซาน@เอ็ม ผลิตภัณฑ์บรรเทาอาการอักเสบบริเวณช่องปากและลำคอ โดยมีส่วนประกอบที่สำคัญคือ ดอกหญ้าหอม (ดอกคาโมไมล์), เพเพอร์มินต์, เชจ ออยล์, เทียนสัตตบงกช, โพนินิดีล ออยล์, เบอร์กามอต ออยล์, ยูคาลิปทอล และ เมทิลซาลิไซเลต คุณสมบัติของคามิลโลซาน จะช่วยในเรื่องการต่อต้านอาการอักเสบ ซึ่งมีส่วนผสมร่วมกันกับการต่อต้านแบคทีเรียของน้ำมันหอมระเหย ซึ่งอยู่ในรูปแบบที่ละลายน้ำ จึงเหมาะสำหรับการอักเสบของบริเวณช่องปากและลำคอ Ref : https://kamillosanmthailand.com/howto.html รายละเอียดและคุณสมบัติเด่น Kamillosan-M Spray เป็นสเปรย์พ่นคอที่มีส่วนผสมจากธรรมชาติ 7 ชนิดและยาปฏิชีวนะ 2 ชนิด ที่ทำงานร่วมกันอย่างมีประสิทธิภาพเพื่อบรรเทาอาการเจ็บคอ ระคายคอ เสี่ยงแหบและลดการอักเสบในช่องปากและลำคอได้อย่างรวดเร็วและปลอดภัย ด้วยสารสกัดจากดอกคาโมไมล์และน้ำมันหอมระเหยหลากหลายชนิด เช่น เปปเปอร์มินท์, เมนทอล และไอบีเก็ก ช่วยให้ความรู้สึกสดชื่น ลดกลิ่นปาก พร้อมดูแลสุขภาพช่องปากและลำคอของคุณได้อย่างครบวงจรเหมาะสำหรับผู้ที่ต้องการบรรเทาอาการอักเสบในลำคอ เหนืออีกเสบ แผลในปาก หรืออาการระคายเคืองที่เกิดจากการสูบบุหรี่หรือพูดมาก Ref : https://vcare-plus.com/kamillosan-m-spray-20ml/	พ่นวันละ 3 ครั้ง บริเวณช่องปากและลำคอ โดยการกดหัวพ่นฉีด 2 ทีต่อครั้ง แนะนำให้ใช้ภายหลังจากรับประทานอาหารแล้ว Ref : https://kamillosanmthailand.com/howto.html	พ่นสเปรย์ 1-2 ครั้ง เข้าไปในลำคอหรือช่องปาก วันละ 3 ครั้ง หรือตามความจำเป็น สามารถใช้ได้ทั้งเด็กอายุ 6 ปีขึ้นไปและผู้ใหญ่ เพื่อผลลัพธ์ที่ดี ควรพ่นหลังจากรับประทานอาหารและก่อนนอน ไม่ควรกินอาหารหรือดื่มน้ำทันทีหลังจากพ่นเพื่อให้ยาออกฤทธิ์ได้เต็มที่ Ref : https://vcare-plus.com/kamillosan-m-spray-20ml/		- ห้ามใช้ในผู้ที่แพ้ประวัติแพ้ส่วนประกอบใดๆ ในผลิตภัณฑ์ โดยเฉพาะดอกคาโมไมล์หรือพืชตระกูล Asteraceae - หลีกเลี่ยงการสัมผัสกับดวงตา หากเกิดอาการแพ้หรือระคายเคือง เช่น ผื่นแดง คัน หรือบวม ควรหยุดใช้และปรึกษาแพทย์ทันที - ไม่ควรใช้ในเด็กอายุต่ำกว่า 6 ปี นอกจากได้รับคำแนะนำจากแพทย์หรือเภสัชกร เนื่องจากยังไม่มีข้อมูลด้านความปลอดภัยที่เพียงพอ - หากอาการไม่ดีขึ้นภายใน 3-5 วัน หรือมีอาการแย่ลง เช่น มีไข้สูง ควรปรึกษาแพทย์ - เก็บผลิตภัณฑ์ในที่แห้งและเย็น อุณหภูมิไม่เกิน 25 องศาเซลเซียส ให้พ้นจากแสงแดดและความร้อน และเก็บให้พ้นมือเด็ก - ควรปิดฝาให้สนิทหลังใช้งานทุกครั้ง Ref : https://vcare-plus.com/kamillosan-m-spray-20ml/
Propoliz mouth spray	โพรโพลิส เม้าท์ สเปรย์ ปราศจากยาปฏิชีวนะหรือยาฆ่าเชื้อที่ทำให้ดื้อยา น้ำตาล แซคคาริน สารกันเสียและเมทิลซาลิไซเลตที่ใช้ติดต่อกัน จะทำให้มีอาการระคายคอกกลับมา โพรโพลิส เม้าท์ สเปรย์ เหมาะสำหรับ ... เมื่อมีอาการเจ็บคอ	วิธีใช้ : พ่นสเปรย์ในช่องปาก ครั้งละ 2-3 พัฟ ใต้อยตามที่ต้องการ	สำหรับน้องอายุตั้งแต่ 3 ปีขึ้นไป สามารถเลือกใช้ Propoliz Mouth Spray ซึ่งสูตร Propoliz Mouth Spray จะมีรสหวานเย็นจากเมนทอล สเปียร์มินท์ เปปเปอร์มินต์ สำหรับน้องอายุตั้งแต่ 6 ปีขึ้นไป สามารถเลือกใช้ได้ทั้งสเปรย์ที่แนะนำข้างต้น และสามารถ	✗	ข้อควรระวัง : หลีกเลี่ยงการใช้ในผู้ที่แพ้ประวัติแพ้เกสรดอกไม้ น้ำผึ้ง ผลิตภัณฑ์จากผึ้งหรือส่วนประกอบอื่นๆ ในตำหรับ และหลีกเลี่ยงการใช้ในผู้ที่แพ้โพรโพลิสได้ โดยแนะนำเป็น Propoliz Kid Mouth Spray

	• ผู้ที่ต้องใช้เสียงหนัก หรือ ใช้เสียงเป็นประจำ • ผู้ที่ทานของทอดของมัน • ผู้ที่ไม่สบายคอ หลังจากรับวัคซีน Ref : https://www.propolizs-pray.com/product/pro-poliz-krachai-mouth-spray-15-ml/		เลือกใช้เป็นเม็ดอมสูตร Propoliz Lozenge Plus และ Propoliz Mixs Lozenge ได้ สำหรับน้องที่อายุ 12 ปี ขึ้นไป สามารถเลือกใช้ผลิตภัณฑ์ทุกชนิดของ โพรโพลิชได้ Ref : https://www.propolizs-pray.com/faq/		หลีกเลี่ยงการใช้ในผู้ที่มีประวัติแพ้ผลิตภัณฑ์จากผึ้ง น้ำผึ้งหรือ ส่วนประกอบของรังผึ้ง (อุบัติเหตุการแพ้ประมาณ < 0.001% ของประชากร) และ ส่วนประกอบที่เกี่ยวข้องในแต่ละสูตรผลิตภัณฑ์ คุณแม่อดัดนิสัยสามารถใช้ผลิตภัณฑ์โพรโพลิชได้ โดยแนะนำมาทส์สเปรย์ Propoliz Mouth Spray เมื่อระยะครรภ์พ้น 3 เดือนแรก หรือไตรมาสแรก คุณแม่ให้นมบุตรสามารถใช้ผลิตภัณฑ์โพรโพลิชได้ โดยแนะนำเป็นมาทส์สเปรย์ Propoliz Mouth Spray
Betadine throat spray	เบตาดีน ไทรอสเปรย์ (Betadine Throat Spray) สเปรย์พ่นปาก และลำคอที่มีคุณสมบัติเป็นแอนตี้เซพติก ด้วยตัวยาสำคัญโพวิโดน ไอโอดีน ซึ่งช่วยลดอาการอักเสบและฆ่าเชื้อได้อย่างมีประสิทธิภาพ ดังนี้ • สเปรย์ฆ่าเชื้อในลำคอ ประสิทธิภาพสูง : ใน 1 มิลลิลิตร มีตัวยาโพวิโดน ไอโอดีน 4.5 มิลลิกรัม จะแยกตัวเป็นไอโอดีนอิสระเพื่อฆ่าเชื้อจุลินทรีย์อย่างรวดเร็ว • สเปรย์พ่นคอ ช่วยแก้เจ็บคอและลดการอักเสบ : ออกฤทธิ์ต้านจุลชีพได้กว้าง ครอบคลุมทั้งเชื้อแบคทีเรีย เชื้อรา และเชื้อไวรัส เพื่อลดอาการอักเสบ • สเปรย์พ่นคอแก้เจ็บคอและดูแลช่องปาก : ช่วยบรรเทาอาการระคายเคือง ความไม่สบายบริเวณลำคอ รวมถึงช่วยดูแลแผลในช่องปาก และช่วยระงับกลิ่นปาก • สเปรย์พ่นคอสำหรับผู้ที่มีอาการทอนซิลอักเสบและแผลในคอ : มีกลไกการปลดปล่อยไอโอดีนอิสระอย่างต่อเนื่อง เพื่อเข้าไปทำลายโครงสร้างของจุลินทรีย์ จึงช่วยลดการติดเชื้อ และการอักเสบบริเวณลำคอ รวมถึงต่อมทอนซิล • ออกฤทธิ์เฉพาะที่และไม่รบกวนการใช้ชีวิตประจำวัน : ด้วยยาออกฤทธิ์เฉพาะบริเวณที่ใช่ และไม่ส่งผลต่อความสามารถในการขับซึ่หรือการทำงานกับเครื่องจักร	A: เมื่อเริ่มมีอาการอักเสบในช่องปากและลำคอ เช่น อาการระคายเคือง มีแผลในช่องปาก หรือมีกลิ่นปาก สามารถใช้สเปรย์พ่นคอ เบตาดีน® ไทรอสเปรย์ เพื่อช่วยบรรเทาอาการได้ โดยมีข้อแนะนำดังนี้ • ครั้งแรกที่ใช้ ให้กดหัวสเปรย์ 4-5 ครั้งบริเวณช่องปากจนมีตัวยาวออกมา • พ่นสเปรย์ 2-3 ครั้งในบริเวณช่องปาก พร้อมออกเสียง "ฮา" ดัง ๆ เพื่อหลีกเลี่ยงการสำลัก ความถี่ในการใช้ : สามารถพ่นฉีดที่บริเวณช่องปากและลำคอได้ทันทีเมื่อมีอาการ (โดยทำตามวิธีพ่นที่ถูกต้องเพื่อหลีกเลี่ยงการสูดสำลักเข้าสู่ทางเดินหายใจ)	ห้ามใช้กับเด็กอายุต่ำกว่า 1 ปี	✗	ข้อควรระวัง : • ห้ามใช้ในผู้มีความผิดปกติของต่อมไทรอยด์ • ห้ามใช้กับผู้ที่แพ้โรคผิวหนังอักเสบ (Dühring's dermatitis herpetiformis) • ห้ามใช้ก่อน ระหว่าง หรือหลังการรักษาด้วยไอโอดีนกับมันดริงส์ • ห้ามใช้กับเด็กอายุต่ำกว่า 1 ปี • ห้ามใช้กับสตรีที่ตั้งครรภ์หรืออยู่ในช่วงให้นมบุตร • ระงับการสูดหรือสำลักระหว่างใช้ยา

Strepsils HHR (Amylmetacresol + 2-4 Dichlorobenzyl alcohol)	สเตริปซิล ยาอมบรรเทาอาการเจ็บคอ 4 รสชาติ รสคูล, รสส้ม และวิตามินซี, รสน้ำผึ้งมะนาว, รสออริจินัล เอชเอชอาร์ 8 เม็ด คุณสมบัติ สเตริปซิล เอชเอชอาร์ เป็นยาอมละลายเสมหะ ช่วยบรรเทาอาการไอ ยาอมบรรเทาอาการเจ็บคอฆ่าเชื้อโรคในปากและลำคอ เป็นยาสามัญประจำบ้าน	วิธีการใช้งาน ผู้ใหญ่และเด็กอายุ 6 ปีขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง			ข้อควรระวัง - หากใช้ติดต่อกัน 2 วันแล้วอาการยังไม่ดีขึ้น ควรปรึกษาแพทย์หรือเภสัชกร - หญิงมีครรภ์และหญิงให้นมบุตร ควรปรึกษาแพทย์หรือเภสัชกรก่อนใช้ยา - ไม่ควรใช้ในเด็กอายุต่ำกว่า 6
Strepsils Maxpro (flurbiprofen)	Symptomatic relief of sore throat https://www.mims.com/philippines/drug/info/strepsils-maxpro/contents	Adults and children over 12 years: Dissolve one lozenge in the mouth every 3 to 6 hours or as prescribed by the physician. Do not take more than 5 lozenges in 24 hours. Duration of therapy should be limited to a maximum of 3 days. If symptoms persist, consult a doctor.	Adults and children over 12 years: Dissolve one lozenge in the mouth every 3 to 6 hours or as prescribed by the physician. Do not take more than 5 lozenges in 24 hours. Duration of therapy should be limited to a maximum of 3 days. If symptoms persist, consult a doctor.		Contraindications Hypersensitivity to flurbiprofen or any of the excipients in the product. Patients who have previously shown hypersensitivity reactions (e.g. asthma, bronchospasm, rhinitis, angioedema or urticaria) in response to acetylsalicylic acid or other NSAIDs. Active, or history of recurrent peptic ulcer/haemorrhage (two or more distinct episodes of proven ulceration) and intestinal ulceration. History of gastrointestinal bleeding or perforation, severe colitis, haemorrhagic or haematopoietic disorders related to previous NSAID therapy. Last trimester of pregnancy Severe heart failure, severe renal failure or severe hepatic failure
Strepsils Maxiplusz (Lignocaine)	ยาอมบรรเทาอาการเจ็บคอ	ผู้ใหญ่และเด็กอายุ 6 ปีขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง	ผู้ใหญ่และเด็กอายุ 6 ปีขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง		หากใช้ติดต่อกัน 2 วันแล้วอาการยังไม่ดีขึ้น ควรปรึกษาแพทย์หรือเภสัชกร หญิงมีครรภ์และหญิงให้นมบุตร ควรปรึกษาแพทย์หรือเภสัชกรก่อนใช้ยา ไม่ควรใช้ในเด็กอายุต่ำกว่า 6 ปี

Strepsils chesty cough (Ambroxol)	ยาอมละลายเสมหะ บรรเทาอาการไอแบบมีเสมหะ	ผู้ใหญ่และเด็กอายุ 6 ปี ขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง	ผู้ใหญ่และเด็กอายุ 6 ปี ขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง	✗	-หากใช้ติดต่อกัน 2 วัน แล้วอาการยังไม่ดีขึ้น ควรปรึกษาแพทย์หรือเภสัชกร - หญิงมีครรภ์และหญิงให้นมบุตร ควรปรึกษาแพทย์หรือเภสัชกรก่อนใช้ยา
Strepsils dry cough (Dextromethorphan)	ยาอมบรรเทาอาการเจ็บคอ ไอ ref. https://banyachalyon.com/product/%e0%b8%82-%e0%b8%a2-11-strepsils-dry-cough-reckitt-box-24s-8s/	ผู้ใหญ่และเด็กอายุ 6 ปี ขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง	ผู้ใหญ่และเด็กอายุ 6 ปี ขึ้นไป โดยอมครั้งละ 1 เม็ด ให้ละลายช้าๆ ในปากทุกๆ 2-3 ชั่วโมง	✗	หากใช้ติดต่อกัน 2 วัน แล้วอาการยังไม่ดีขึ้น ควรปรึกษาแพทย์หรือเภสัชกร หญิงมีครรภ์และหญิงให้นมบุตร ควรปรึกษาแพทย์หรือเภสัชกรก่อนใช้ยา ไม่ควรใช้ในเด็กอายุต่ำกว่า 6 ปี
ยาน้ำแก้ไอผสมมะขามป้อม ทรายทอง 120 ml (ใน 100 มิลลิกรัมเตรียมจากผลมะขามป้อม 18.9084 กรัม ลูกมะแว้งต้น 0.0714 กรัม และตัวยาอื่นๆ)	• บรรเทาอาการไอ ขับเสมหะ และช่วยให้ชุ่มคอ Ref : https://www.abhaishop-com.translate.google.com/product/399288?_x_tr_sl=th&_x_tr_tl=en&_x_tr_hl=en&_x_tr_pto=sc	• รับประทานครั้งละ 2 ช้อนชา วันละ 3-4 ครั้ง เมื่อมีอาการ เข้ายาวดก่อนใช้ยา		✗	• ไม่แนะนำให้ใช้ในเด็กอายุต่ำกว่า 2 ขวบ และหญิงตั้งครรภ์ เนื่องจากมีแอลกอฮอล์ผสมอยู่ 0.2% • ผลข้างเคียง : อาจจะเป็นยาระบายอ่อนๆ
ยาแก้ไอน้ำดำ (มีสารสกัดชะเอมเทศ 0.6 ml/5ml, antimony potassium tartrate 1.2-2 mg/5ml, camphorated opium tincture 0.6 ml/5ml)	• บรรเทาอาการไอ ขับเสมหะ Ref : https://ndi.fda.moph.go.th/uploads/core/a869cd1d8781acde274ba27fb8636f7d.pdf	กินครั้งละ 1-2 ช้อนชา (5-10 ml) กินยานี้วันละ 3-4 ครั้ง เมื่อมีอาการไอ	เด็ก 6-12 ปี: กินครั้งละ ครั้ง ถึง 1 ช้อนชา (2.5-5 ml) กินยานี้วันละ 3-4 ครั้ง เมื่อมีอาการไอ	✗	• ห้ามกินยานี้ติดต่อกันเกิน 3 วัน เนื่องจากทำให้เกิดอาการติดยาและติดยาได้ • ยานี้มีแอลกอฮอล์ จึงห้ามใช้ในหญิงตั้งครรภ์ ผู้สูงอายุ และเด็กอายุต่ำกว่า 6 ปี • อาการข้างเคียง : ปวดหัว เวียนหัว ง่วงซึม นอนไม่หลับ ปาก/คอแห้ง คลื่นไส้อาเจียน ท้องผูก เบื่ออาหาร ปัสสาวะออกน้อยหรือปัสสาวะลำบาก
Betadine gargle 30 ml	• แก้วเจ็บคอ ใช้สำหรับดูแลอาการอักเสบบริเวณช่องปากและลำคอ โดยมีตัวยาโพวีโดน-ไอโอดีน ช่วยฆ่าเชื้อจุลินทรีย์ และต่อต้านเชื้อแบคทีเรีย ซึ่งเป็นหนึ่งในสาเหตุที่อาจทำให้เกิดอาการเจ็บคอ Ref: https://th.betadine.glo	• หยดเบตาดีน® การกลืน 1-2 ชีชี ลงถ้วยผสม เติมน้ำสะอาดลงถ้วยผสม จนครบ 30 ชีชี อมกลั้วคอ 30 วินาที แล้วบ้วนทิ้ง วันละ 2-3 ครั้ง		✗	• เด็กสามารถใช้ยากลั้วคอได้เมื่อสามารถอมกลั้วคอและบ้วนทิ้งเองได้ โดยควรมีผู้ปกครองดูแลขณะใช้ทุกครั้ง และหลีกเลี่ยงการใช้ในเด็กอายุต่ำกว่า 1 ปี

	bal.th/th/product-category/product/cold-and-flu/betadine-gargle			
Diffiam Lozenges	1.Diffiam Anti-inflammatory,Anti-bacterial Lozenges ยามอมแก้อักเสบและต้านแบคทีเรีย Diffiam ผสานสารด้านการอักเสบเบนซิดามีนเข้ากับสารฆ่าเชื้อเพื่อช่วยบรรเทาอาการปวดและช่วยกำจัดเชื้อโรคที่เป็นสาเหตุของการติดเชื้อเล็กน้อยในลำคอและช่องปาก ช่วยบรรเทาอาการปวดบวม และแดงจากอาการเจ็บคอและโรคในช่องปากได้อย่างรวดเร็ว รวมถึงโรคคออักเสบ ต่อมทอนซิลอักเสบ แผลในปาก และหลังจากการทำฟัน การผ่าตัดในช่องปากหรือลำคอ ช่วยต่อสู้กับการติดเชื้อเล็กน้อยในลำคอและช่องปาก เบนซิดามีนช่วยลดอาการบวมและปวด น้ำยาฆ่าเชื้อช่วยกำจัดเชื้อโรคที่อาจทำให้เกิดการติดเชื้อในลำคอเล็กน้อย รูปแบบเม็ดอมช่วยเพิ่มความชุ่มชื้นในช่องปากและลำคอ เพื่อบรรเทาอาการระคายเคือง* รสชาติ น้ำผึ้งและมะนาว รสเบอร์รี่ ยูคาลิปตัสและเมนทอล Ref : https://diffiam.sg/product/diffiam-anti-inflammatory-anti-bacterial-lozenges/	ผู้ใหญ่และเด็กอายุตั้งแต่ 6 ปีขึ้นไป ขนาดและวิธีใช้: อมครั้งละ 1 เม็ด ทุกๆ 1 - 2 ชั่วโมงตามความจำเป็น (สูงสุดไม่เกิน 12 เม็ดต่อวัน) ค่อยๆ อมยาให้ละลายในปากช้าๆ ทีละเม็ด ห้ามเคี้ยว ถ้าอาการเจ็บคอมีสาเหตุจากการติดเชื้อ ควรให้ยาด้านเชื้อแบคทีเรียที่เหมาะสมร่วมกับยายามอม Diffiam	ระวังการใช้ในผู้ป่วยที่มีการทำงานของตับและไตบกพร่อง	ห้ามใช้ หากแพ่งพอลียส์ (Blister) ขารัดหรือฉีกขาด ห้ามใช้ หากคุณมีอาการแพ้สารเบนซิดามีน (Benzylamine) หรือยาต้านการอักเสบชนิดอื่นๆ หากเกิดอาการแพ้ ให้หยุดใช้ยาทันทีและรีบไปพบแพทย์ ระหว่างที่ใช้ยานี้: เก็บยาให้พ้นมือเด็ก และไม่แนะนำให้ใช้ในเด็กอายุต่ำกว่า 6 ปี ไม่ควรใช้ยาดูติดต่อกันเกิน 7 วัน หากอาการยังไม่ทุเลาลง ควรปรึกษาแพทย์หรือเภสัชกร ยานี้มีส่วนผสมของไอโซมอลต์ (Isomalt) 31 กรัมต่อ 12 เม็ด (ซึ่งเป็นปริมาณสูงสุดที่แนะนำต่อวัน) การบริโภคผลิตภัณฑ์ที่มีส่วนผสมของไอโซมอลต์มากเกินไปอาจมีฤทธิ์เป็นยาระบายได้ (ทำให้เกิดท้องเสีย) โปรดปรึกษาแพทย์หากคุณมีปัญหาเกี่ยวกับตับหรือไต กำลังตั้งครรภ์หรือกำลังให้นมบุตร ข้อควรระวัง: ระวังการใช้ในผู้ป่วยที่มีการทำงานของตับและไตบกพร่อง ข้อห้ามใช้: - ผู้ที่มีประวัติแพ้ยาหรือยากลุ่ม NSAIDs - สตรีตั้งครรภ์ไตรมาสสุดท้าย - ผู้ที่มีเลือดออกในกระเพาะอาหารหรือกระเพาะอาหารทะลุ - โรคตับหรือโรคไตรุนแรง - ผู้ป่วยใช้เลือดออก - ยานี้อาจเพิ่มความเสี่ยงต่อการเกิดเลือดออกหรือแผลในทางเดินอาหาร เพิ่มความเสี่ยงต่อการตีบตันของหลอดเลือดสมองและหลอดเลือดหัวใจ - ยานี้อาจทำให้เกิดภาวะบวม - ระวังในผู้สูงอายุหรือ

				มีความดันโลหิตสูง
	2.Diffiam Anti-inflammatory Lozenges ยาอมแก้อักเสบ Diffiam มีส่วนประกอบของเบนโซไคตามีน ซึ่งได้รับการพิสูจน์ทางการแพทย์แล้วว่าช่วยบรรเทาอาการปวดได้1 ด้วยการมุ่งเป้าไปที่การอักเสบซึ่งเป็นสาเหตุของความเจ็บปวด ยาอมนี้จึงช่วยบรรเทาความเจ็บปวด บวม และแดงจากอาการเจ็บคอ และโรคในช่องปากได้อย่างรวดเร็ว รวมถึงโรคคออักเสบ ต่อมทอนซิลอักเสบ แผลในปาก และหลังจากการทำฟัน การผ่าตัดในช่องปากหรือลำคอ ยาด้านการอักเสบจะออกฤทธิ์ที่ต้นเหตุของอาการเจ็บคอ รูปแบบเม็ดอมช่วยเพิ่มความชุ่มชื้นในช่องปากและลำคอ เพื่อบรรเทาอาการระคายเคือง* รสนชาติ มินต์ Ref : https://diffiam.sg/product/diffiam-anti-inflammatory-lozenges/	อายุ: ผู้ใหญ่และเด็กอายุตั้งแต่ 6 ปีขึ้นไป ขนาดและวิธีใช้: อมครั้งละ 1 เม็ด ทุกๆ 1 - 2 ชั่วโมงตามความจำเป็น (สูงสุดไม่เกิน 12 เม็ดต่อวัน) ค่อยๆ อมยาให้ละลายในปากช้าๆ ทีละเม็ด ห้ามเคี้ยว	ระวังการใช้ในผู้ป่วยที่มีการทำงานของตับและไตบกพร่อง	ห้ามใช้ หากแพงฟลอยส์ (Blister) ข่ารดหรือฉีกขาด ห้ามใช้ หากคุณมีอาการแพ้สารเบนซิดามีน (Benzylamine) หรือยาต้านการอักเสบชนิดอื่นๆ หากเกิดอาการแพ้ ให้หยุดใช้ยาทันทีและรีบไปพบแพทย์ ระหว่างที่ใช้ยานี้: เก็บยาให้พ้นมือเด็ก และไม่แนะนำให้ใช้ในเด็กอายุต่ำกว่า 6 ปี ไม่ควรใช้ยาดติดต่อกันเกิน 7 วัน หากอาการยังไม่ทุเลาลง ควรปรึกษาแพทย์หรือเภสัชกร ข้อควรระวัง: ระวังการใช้ในผู้ป่วยที่มีการทำงานของตับและไตบกพร่อง ข้อห้ามใช้: - ผู้ที่มีประวัติแพ้ยาหรือยาในกลุ่ม NSAIDs - สตรีตั้งครรภ์ไตรมาสสุดท้าย - ผู้ที่มีเลือดออกในกระเพาะอาหารหรือกระเพาะอาหารทะลุ - โรคตับหรือโรคไตรุนแรง - ผู้ป่วยไข้เลือดออก - ยานี้อาจเพิ่มความเสี่ยงต่อการเกิดเลือดออกหรือแผลในทางเดินอาหาร เพิ่มความเสี่ยงต่อการตีบตันของหลอดเลือดสมองและหลอดเลือดหัวใจ - ยานี้อาจทำให้เกิดภาวะบวม - ระวังในผู้สูงอายุหรือมีความดันโลหิตสูง
Diffiam forte throat spray	ดิฟแฟม ฟอर्ट สเปรย์ แก้อักเสบ สเปรย์พ่นคอ DIFFLAM Forte บรรเทาอาการอักเสบ ช่วยบรรเทาอาการได้อย่างรวดเร็วและตรงจุดสำหรับบริเวณที่อักเสบและเจ็บปวดในช่องปากและลำคอที่เข้าถึงยาก	วิธีใช้สเปรย์พ่นคอ DIFFLAM Forte บรรเทาอาการอักเสบ • สำหรับผู้ใหญ่และเด็กอายุ 12 ปีขึ้นไป: พ่นสเปรย์ 2-4 ครั้งลงบนบริเวณที่เป็นแผลหรืออักเสบโดยตรง แล้วกลืนเบาๆ ทำซ้ำทุก 1.5 ถึง 3 ชั่วโมงตามความจำเป็น • สำหรับเด็กอายุ 6 ถึง 12 ปี: พ่นสเปรย์ 2 ครั้งลงบนบริเวณที่เจ็บหรืออักเสบโดยตรง แล้วกลืนเบาๆ ทำซ้ำทุก 1.5 ถึง 3 ชั่วโมงตามความจำเป็น	ระวังการใช้ในผู้ป่วยที่มีการทำงานของตับและไตบกพร่อง	ห้ามใช้ หากคุณมีอาการแพ้สารเบนซิดามีน (Benzylamine) หรือยาต้านการอักเสบชนิดอื่นๆ หากเกิดอาการแพ้ ให้หยุดใช้ยาทันทีและรีบไปพบแพทย์

	ระบบการส่งยาแบบใหม่ ช่วยให้ออกฤทธิ์เฉพาะที่ แม้กระทั่งบริเวณด้านหลังลำคอ และบริเวณอื่นๆ ที่เข้าถึงยากในลำคอและช่องปาก สเปรย์พ่นคอ DIFFLAM Forte บรรเทาอาการอักเสบ ด้วยการส่งสารออกฤทธิ์ในปริมาณที่แม่นยำและตรงจุด ช่วยบรรเทาอาการอักเสบและปวดเฉพาะที่ได้อย่างรวดเร็ว ภายใน 60 วินาที สูตรปราศจากน้ำตาล กลูเตน และแลคโตส พร้อมรสชาติสดชื่น ลองเลือกใช้สเปรย์พ่นคอ DIFFLAM Forte Anti-inflammatory Throat Spray เมื่อคุณต้องการบรรเทาอาการแดง บวม ปวด และอักเสบในช่องปากหรือลำคอชั่วคราว Ref : https://diffiam-sg.translate.google.com/product/diffiam-anti-inflammatory-lozenges/?_x_tr_sl=en&_x_tr_tl=th&_x_tr_hl=th&_x_tr_pto=sg	ขนาดและวิธีใช้: • อายุมากกว่า 12 ปี: พ่น 2-4 ครั้ง ทุกๆ 1.5 - 3 ชั่วโมงตามความจำเป็น • เด็กอายุ 6-12 ปี: พ่น 2 ครั้ง ทุกๆ 1.5 - 3 ชั่วโมงตามความจำเป็น • พ่นสเปรย์ไปยังบริเวณที่มีอาการโดยตรง		ระหว่างที่ใช้ยา: เก็บยาให้พ้นมือเด็ก และไม่แนะนำให้ใช้ในเด็กอายุต่ำกว่า 6 ปี หากอาการยังไม่ทุเลาลง ควรปรึกษาแพทย์หรือเภสัชกร ไม่ควรใช้ติดต่อกันเกิน 7 วัน เว้นแต่จะอยู่ภายใต้การดูแลของแพทย์
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ข้อห้ามใช้

ชื่อสามัญ/ชื่อการค้า	ข้อบ่งใช้	Dose ผู้ใหญ่	Dose เด็ก	Dose ปรับตามตบ/ใด	ข้อห้ามใช้/การระวัง
Solmax capsule	Indications/Uses Mucolytic agent for relief of cough associated with acute and chronic bronchitis, asthma, bronchiectasis and emphysema. ref.https://www.mims.com/thailand/drug/info/solmax-solmax-forte-solmax-kids?type=full	Dosage/Direction for Use Solmax Capsule: Adult and children >12 yr 1 capsule 3 times a day after meal.	Dosage/Direction for Use Solmax Capsule: Adult and children >12 yr 1 capsule 3 times a day after meal.	✗	Contraindications Contraindicated in patients with active peptic ulcer or with known hypersensitivity to Carbocisteine. Warnings Administer with caution in patients with history of gastric or duodenal ulcer. Administer to infants only as advised by the physician. Use in Pregnancy: Do not recommend the use of Carbocisteine in the first-trimester of pregnant. Special Precautions Administer with caution in patients with history of gastric or duodenal ulcer. Since the effects of

					Carbocisteine when used during pregnancy or lactation are not known, it should be administered with caution to women during these periods. Administer to infants only as advised by the physician.
Muclear	Indications/Uses Acetylcysteine is used as a mucolytic agent in the adjuvant treatment with abnormal, viscid, or inspissated mucus secretion in such conditions as acute and chronic bronchopulmonary disorder (e.g., pneumonia, bronchitis, emphysema, tracheobronchitis, chronic asthmatic bronchitis, tuberculosis, bronchiectasis, primary amyloidosis of lung). Prevent or lessen hepatic injury caused by Paracetamol overdose. ref.https://www.mims.com/thailand/drug/info/muclear?type=full	Dosage/Direction for Use For mucolytic effects: Adults and children over 12 years: Dissolve 1 tablet in a half glass of water and drink after dinner daily.	Dosage/Direction for Use For mucolytic effects: Adults and children over 12 years: Dissolve 1 tablet in a half glass of water and drink after dinner daily.		Contraindications Hypersensitivity or intolerance to any component of the product. Special Precautions Should be used with caution in patient with phenylketonuria (a hereditary metabolite disease) because MUCLEAR contains aspartame as a source of phenylalanine. Should be used with caution in asthmatic patients. It should also be used with caution in patient with a history of peptic ulcer disease, both because drug-induced nausea and vomiting may increase the risk of gastrointestinal haemorrhage in patients predisposed to the condition, and because of theoretical risk that mucolytics may disrupt the mucosal barrier. Use In Pregnancy & Lactation Should not use during pregnancy and lactation without medical advice.